Speaking Notes: Imaging Center Lunch

> ### 📌 UPDATED 2026-11-03 EVENING (Rick's review items R9 to R19; LUNCH, Thursday 2026-11-05). Earlier the same day: Rick's answers to the 11-02 round. Earlier updates 2026-11-02 (review round), 2026-11-01 (RadAssist on slide 18) and 2026-10-22 (Luis rehearsal).
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> **In the room:** Luis, Fernando Escarzaga, and **Jesse Martinez** (an engineer; quiet, analytic, humble). Format: a quiet place, a conversation, not a pitch to an audience. Rick has court by Zoom on Wed 10-28.
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> **FINAL STRUCTURE, Rick-approved 2026-11-03 20:46 , corrected 20:49 (35 slides; 28 in the no-numbers deck).** Both Part dividers STAY (7 and 11). The hours slide and the 95% slide are one slide, **8 "The problem: what prior auth costs you"** (one headline number, three lines, one closing line). Radiology report assistance moved to the appendix (**33**); the one line that stays in the pitch is on slide 18 (the signed report flows into the RIS). The engagement (**23**), the timeline (**24**) and what I need from you (**25**) now come BEFORE the Thank You (**26**), so the price is reached at about 34:30 and the close ends at 40:00. The appendix divider (**27**, "Reference, only if asked") is followed by the shortlist (28), running costs (29), the opening checklist (30), ideas worth a look (31, 32) and report assistance (33). Phase 4 deliverable reworded (R26: every system, bought and built, set up, connected and passing end-to-end tests; vendors configure their own products, Rick manages them, connects everything and runs the tests). Phase 1 time is with ALL THREE owners, Dr. Ramos, Fernando and Jesse (R27; Jesse named for the technical and integration decisions). Phase 1 make-or-buy is per REQUIREMENT (R28: met by a system you buy, or a piece Rick builds, with a ballpark running cost for anything built). A two-page lunch card (`Imaging-Center-LUNCH-CARD-2026-11-05.pdf`) replaces the folded sheet as the pocket card.
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> **Rick's evening review, 2026-11-03, applied:** - **Support plan, APPROVED 17:54:** $3,000 a month for up to 20 hours, $150 an hour beyond, from June 2027, 12-month term with a 90-day check-in, separate from the $45,000; on slide 25 (one row), slide 31 (support row), the leave-behind, slide 25 notes and Q&A Q16. **Two more appendix slides:** 34 "Opening checklist" (required to open, outside the fee, owners named in Phase 1) and 35 and 36 "Ideas worth a look" (follow-up recommendation tracking leads; three smaller ideas on 36). - **Two new appendix slides.** Slide 30, "Systems to select in Phase 2: starting shortlist" (RIS and PACS, dictation, billing and clearinghouse, from Sherlock's landscape; a starting shortlist, the final pick is Phase 2). Slide 31, "What it costs to keep running, and where it runs" (running costs are not in the $45,000; three hosting options). Both are reference, only if asked, inside the last ten minutes. - **Phase 2 now names every system** (slide 25, 27, leave-behind): (1) scheduling; (2) RIS and PACS, evaluated both as one bundled cloud system and as separate best-of-breed RIS plus PACS (Exa is one example); (3) prior authorization and eligibility; (4) patient phones, IVR and AI answering; (5) radiologist voice recognition and dictation; (6) billing and clearinghouse; (7) imaging-specific: modality integration and worklist, reading workstations, image sharing (CD and cloud) (trimmed by Rick 17:48: owners' asks first, then imaging-specific; generic IT items removed). - **The scorecard (slide 17) says what each piece does**, with records split into inbound requests and outbound priors; the "why" lives in the notes. It is now "three of the eight." - **RIS and PACS position:** one cloud system for both at a single center; the lunch line is under slide 17 and in the Fernando Q&A. - **Slide 20:** the HB 300 line is Rick's plain version; the TCPA line ends "I've built per-user limits like that in my own production systems, and this one gets designed in from day one." (Rick 21:44); a "Talk it through" block per boundary. - **Slide 25:** priced for one center with one selected system per category; running costs after the build are outside the fee, in the center's own accounts, at cost, with a monthly statement; the Phase 3 deliverable "Total cost of ownership and operating budget" (three hosting options). The three-centers answer is replaced.

What changed on 2026-11-02 and earlier (slide numbers): - **New slide 6, "What an AI agent is."** Plain language: the two kinds, the guardrails. - **New slide 10, one order through prior authorization,** with the working demo already built. **Alan's demo is live at https://rfinlay.github.io/imaging-priorauth-demo/** and is linked on slide 10 and leave-behind page 2. Click through it yourself Thursday morning. - **New slide 24, "What the dates depend on."** The assumptions. The only place a date is promised, and every date on it is conditional. - **Timeline:** opening targeted around **June 2027, April at the earliest**. The **build starts around March 2027** so prior authorization works before opening. Say only "your new center." **Never mention any building, property, address, or what was at the site before.** - **Pricing, APPROVED by Rick 2026-11-02 (slides 25 to 27, before the Thank You):** one fixed-fee engagement for the new center, **$45,000 in five phases, November 2026 to May 2027**, ready for a ~June 2027 opening with about a month of buffer. Phase 1 Discovery and Requirements $7,000 (Nov to mid-Dec; $4,000 at signing, $3,000 on delivery) · Phase 2 Vendor and System Selection $8,000 (mid-Dec to Jan) · Phase 3 Design and Staffing Plan $7,000 (Feb) · Phase 4 Build and Configure $14,000 (Mar to Apr; $7,000 March 1, $7,000 April 1) · Phase 5 Testing, Training and Go-Live $9,000 (May). Phases 2, 3 and 5 are paid in full at the start. Each phase starts only after the previous deliverable is signed off. Outside the phases: $150/hr, approved first, against a retainer they top up. The timeline flexes if the opening moves; the price does not. **Slide 26 is the timeline (Gantt) and slide 27 is "What I need from you."** The old design/build split and every red "Proposed figures" tag are gone. - **Slide 18 is a spoken RadAssist OVERVIEW, no live demo (Luis's feedback, applied 2026-11-03).** One minute on the slide: what it is, Luis uses it daily, his templates and his rules, it drafts and he signs, four modules (calcium, thyroid, spine MRI, CT spine), nothing leaves the browser, not a device. Then hand the table to Luis. The Spine MRI demo walkthrough stays in archive/DEMO-SCRIPT.md Part A, marked not used Thursday; the prior authorization demo on slide 10 (Part B) is unchanged. Roadmap: voice dictation, then brain and cardiac CTA (CAD-RADS). AI image auto-fill is "a future item with a regulatory path," never a promise. **Selling RadAssist to other centers stays OUT of the room** (see slide 18). - **No license terms at the table (Rick, 2026-11-03).** What is being sold is a scope of work: a design for one imaging center that can be put anywhere. Ownership of any product pieces is settled in the contract, not at lunch; the center owns its configuration and its data. No arrangement of Luis's is described anywhere. IF ASKED "who owns what?" is under slide 21. - **If they float a full-time role (Rick, 2026-11-03):** IF ASKED block under slide 25. "I'd want to hear it. Let's finish Phase 1 first; that's the job to judge me on." No numbers. - **LOCKED 2026-11-02: lunch, Thursday 2026-11-05, with Luis, Fernando Escarzaga and Jesse Martinez.** Be ready Thursday morning; the prep plan in Part 4 runs Tuesday through Thursday morning (Rick-approved 2026-11-03). - **Transitions (Rick, 2026-11-02):** anything you say before a slide comes up is the last block of the previous slide's notes, headed **LEAD-IN TO SLIDE N (say before you advance)**, so it is never confused with that slide's own THEN SAY. - **Exa answer** is on slide 10: vendor-neutral, never disparage. - **Late 11-02 items (R5e, R6b, R7):** slide 35's last block is now the deployment default (one environment under one contract, each organization in its own isolated instance with its own database, keys, credentials and audit log; fully separate stacks only for separately owned centers that want separate hosting; never one shared database). The HB 300 depth block under slide 20 carries the statute's verified numbers. Lunch is about an hour, so the slide clock is 40 minutes and a skip order follows below.

Fernando, per Luis: straightforward, tough, tight with money, the best administrator Luis has worked with in 25 years. Distrustful after vendors over-promised. Visual: expect "When can I see it working?" He will have his own ideas (AI for prior-record retrieval, for one). **Give him the floor.**

Luis asked how you make money. One sentence: "One fixed fee, in writing, for taking the new center from requirements to opening day. Anything outside it is approved first and billed by the hour."

Process facts from Luis, use them: outside priors arrive by courier on CD or DVD, and after five days the study is read without them. On the calcium score slide it is "calcium," never "nodules."

El Paso, lunch, Thursday 2026-11-05, with Luis, Fernando Escarzaga and Jesse Martinez. Lunch is about an hour. The slides get 40 minutes of it. 35 slides (28 in the no-numbers deck), plus the prior authorization demo he can click on slide 10. RadAssist on slide 18 is a spoken overview, not a demo.

The hour, budgeted (Rick, 2026-11-02). Ten minutes before the first slide: sitting down, ordering, the cold-open card (Part 1). Forty minutes of slides: the clock on every slide header below runs from 0:00 at slide 1 to 40:00 at the Thank You. The price is reached at about 34:30, before the Thank You. Ten minutes after: Fernando talking, questions, and the appendix if the room asks. If the food arrives mid-deck, stop at the end of the slide you are on and eat; the clock pauses, the cut list below does not change.

If short on time, skip, in this order (final order, 2026-11-03). The clock is 40 minutes and you will not get all of it. Cut in this order and nothing you need is lost: **slide 16** (owner numbers; the scorecard on 17 names it) first, then **slide 23** (if it were my money; keep its closing question and ask it on the Thank You, 28), then **slide 13** down to one sentence ("Your RIS will do reminders. Let it."), then **slide 8** down to its headline number and its last line ("A process that approves 95% of what it touches is one you automate"), then **slide 18** down to its first two sentences and Luis's Tuesday. **Never skip 3, 4, 6, 10, 17, 18, 21, 22, 24 or 25.** If you reach 30:00 on the slide clock and are not yet on slide 21, jump to 21, then 24 and 25: the bus answer, the assumptions, the price. (The report-assistance slide is in the appendix now and is never on the clock.) **In the room:** Dr. Luis Ramos (uses RadAssist daily; your champion), Fernando Escarzaga (founded Southwest X-Ray and sold it in 2019; the skeptic and the signer), and Jesse Martinez (an engineer; quiet and analytic).

This is a rehearsal sheet, not a reference. Read it aloud, top to bottom, in order. The RUNBOOK is for lookup. This is for practice.

Every acronym and term of art in this sheet is defined in Part 5, the glossary, on the last pages. Flip to the back.

How each slide block works:

- **SAY** is the spoken line. Read it aloud until it sounds like you and not like a page.
- **THEN SAY** is the caveat. It is its own line so you say it unprompted, never as an afterthought when challenged.
- **DEPTH / IF ASKED** is what stands behind the claim: the supporting figures, the source and its limits, what the number is not. Know it. Do not recite it.
- **BEAT** is the physical instruction: where to pause, where to stop, where to write.
- **LEAD-IN TO SLIDE N (say before you advance)** is the transition: the question or line you say on the current slide, before the next one comes up. It is always the last block of a slide's notes.

PART 1: THE COLD-OPEN CARD

The first 90 seconds, verbatim

"Luis, Fernando, thank you for lunch.

One sentence on who I am. I build and operate agent systems, and Luis uses one of them every day on real patients. That is the only credential that matters today.

I am not here to sell you software today. I am here to tell you which decisions you are about to make that are free right now, and will not be free by the time you open.

Before I start, one thing I would rather hear from you than secondhand. Luis told me you built and sold imaging here in El Paso before this. What did you build, and how many centers were you running at the end?"

Then stop. Let him answer. Write down the numbers he gives you. Everything you say about his history for the rest of the lunch comes from that answer, not from a file.

Names

- **Fernando.** Never Jesus. His legal first name is Jesus and every business source uses Fernando.
- **Spell it Escarzaga.** Not Escargaza, not Escarcega. Both appear in public records and both are wrong.
- **Veronica Escarzaga** was a named co-seller of Southwest X-Ray. He may not be the only decision-maker. Do not discover her at the table.

The landmine strip: never say these

Never say	Why, in five words
The Akumin lawsuit, in any form	Active litigation. Knowing it reveals digging.
Desert Imaging	Different man, different company.
Scale, platform, roll-up, synergies	The vocabulary of the buyer.
"Density in key markets"	Near-verbatim from the buyer's press release.
"When you sold to private equity"	He sold to a public company.
Akumin or Stonepeak as a reference or model	Never praise the buyer.
Headcount reduction, displaced FTEs, layoffs	He knows the people who answer phones.
His founding year	Three sources, three years. Let him say it.
His age	Never.
"Jesus"	Fernando.
"VA requires precertification"	Dropped 2026-09-08. Referral and SEOC govern.
"TRICARE prior-auths MRI, CT, PET"	False outside Prime. Imaging not listed.
"185,499 retirees live in El Paso"	Installation access. Wider than county.
Any share of his imaging VOLUME by payer	Coverage shares only. He converts.
"You open in June" as a fact	Say "your target, around June, April at the earliest."
Any building, property, address, or what was at the site	Say "your new center." Nothing more.
Selling RadAssist to other centers	Private, with Luis only. Never in front of Fernando.
License terms, or any arrangement of Luis's	Scope of work only. The contract settles ownership.
Anything negative about Exa	Vendor-neutral. "If theirs is good, use it."

If he raises any of it himself: listen, say little, ask nothing, move on.

The reframe behind every line today: he is not looking to grow. He is reclaiming operational control and building lean on purpose. Say "run this center with a lean front office." Never say "grow."

PART 2: THE 35 SLIDES, IN ORDER

SLIDE 1 · Six months of free decisions · 0:00 to 1:00

SAY

The cold-open card above, word for word. The framing line is the one to get right:

"I am not here to sell you software today. I am here to tell you which decisions you are about to make that are free right now, and will not be free by the time you open."

DEPTH / IF ASKED

Nothing to defend here. The only claim is that Luis uses RadAssist daily on real patients, and Luis is sitting there.

BEAT

Deck on slide 1 while they sit down. Say the framing line slowly. Ask the "what did you build" question before you advance. Write his answer down. If he names several companies, he has built and run this kind of business more than once. Do not explain to him what a RIS, a billing system, or an interface is at any point today; he knows. Explain only what the agents do.

SLIDE 2 · Where you are · 1:00 to 2:30

SAY

"Here is where you are, as I understand it. The MRI is signed. Nobody is hired, and no systems are chosen.

Today, every operational decision you make is cheap, and you can reverse it.

On January first, the federal prior authorization APIs go live at the payers.

Around March, the build starts, so prior authorization is working before you open.

Your target is to open your new center around June, April at the earliest. Whatever is in the vendor contracts by then is what you live with.

Greenfield is the entire argument today. By the time you open there will be a RIS, a billing vendor, a phone system, and people hired around whatever those systems cannot do. Nothing I am going to show you can be done this cheaply after you open."

DEPTH / IF ASKED

The January date is CMS-0057-F, the Interoperability and Prior Authorization Final Rule, 89 FR 8758, published 2024-02-08. Compliance dates were verified in the rule text itself: decision timeframes and public reporting from 2026-01-01, the FHIR Prior Authorization API and the Patient Access, Provider Access, and Payer-to-Payer APIs by 2027-01-01. It binds payers, not providers, and covers Medicare Advantage, Medicaid and CHIP, and the federally facilitated exchange plans. Do not expand on it here. It gets its own slide at 8.

Who "the payers" are, in plain terms: the organizations that pay the center for each study. For a new El Paso imaging center that is five groups. Commercial insurers, meaning employer and individual plans from companies like Blue Cross, UnitedHealthcare, and Aetna. Medicare, in two forms: traditional Medicare, run by CMS directly, and Medicare

Advantage, where a private insurer runs the Medicare plan under contract. Medicaid and CHIP, the state-run programs for low-income adults and children. The federal exchange plans, meaning the individual coverage people buy on healthcare.gov. And self-pay, meaning the patient. The federal rule on slide 9 binds only Medicare Advantage, Medicaid and CHIP, and the exchange plans. It does not bind commercial employer plans or traditional Medicare, and it does nothing for self-pay. That is why the payer mix question on slide 9 matters: the more of his volume that bills to the plans under the rule, the more the January date is worth to him.

BEAT

Say the obvious thing plainly. Do not oversell the January date yet.

LEAD-IN TO SLIDE 3 (say before you advance)

"Which RIS and billing vendors are you looking at, and have you signed anything yet?" The answer tells you whether the next slide is still free.

SLIDE 3 · The free move · 2:30 to 4:30

SAY

"Whatever you decide about me, do this. Put four things in every vendor contract before you sign. They cost nothing today.

One. API access to your own data, named in the contract. Not 'available on request.'

Two. The HL7 interface count and the fee schedule, fixed at signature. Not quoted per interface later.

Three. Data export rights, in standard formats, on demand, at no charge.

Four. DICOM query permissions, worklist, query and retrieve, DICOMweb, for the systems you authorize.

Before signature these are contract terms. After signature they are hostage terms. I will give you the exact language."

DEPTH / IF ASKED

Why DICOM specifically: it is the one standardized surface that does not depend on vendor goodwill. HL7 version 2 is what the RIS will actually speak. FHIR on the RIS side: do not pay extra for it, and do not plan around it. The FHIR that matters is on the payer side, by federal rule, and it is mandated there. On the RIS side it is not mandated, and at a small vendor it is often partial, extra-cost, or a roadmap item.

If a shortlisted RIS vendor claims a FHIR capability, the rider to term two: "Name it and demonstrate it before signature, at no additional interface fee." That is not a requirement to have FHIR. It is a requirement not to be charged later for something they claimed in the sales cycle. Fernando will recognize the move.

Interface fees are customary and quoted per interface. That sentence is the whole statement. A dollar range for HL7 interfaces circulates widely and could not be verified at any primary source. Do not quote one.

Interface complexity is a Phase 2 deliverable (vendor and system selection), not an assumption. "The count and the fee schedule" is the contract protection; knowing which interfaces you actually need, and how hard each one is, is the work. Phase 2 produces that list: every interface the automation plan requires, whether it runs over HL7 version 2 or an API, which direction the data moves, and what the vendor charges for each.

What "API access to your own data" has to cover, if he asks you to be specific: read access to the clinical and scheduling data the center owns, the ability to export it in bulk on demand, the audit and error logs (you cannot operate what you cannot see fail), and a test or sandbox environment so changes are not proven in production. Name the four in the contract language rather than the phrase "access." On Rick's own build the question answers itself: the

center-specific code, its logs, and its database live in a repository and an environment under the center's organization, so that access is theirs by construction.

Why export rights matter more than they used to: Mirth Connect, the usual free HL7 engine, went closed-source commercial at version 4.6 in March 2025. The last open release is 4.5.2 from 2024-09-23, unmaintained, which is a security consideration on a path carrying patient data. Verified at the GitHub releases API. You cannot assume a free integration engine anymore, so the data has to be reachable by contract. (Research brief, sections 5.1 and 5.2.)

BEAT

This is the slide that proves you are on their side of the table, and it does not matter whether they hire you. Fernando has negotiated these contracts. Watch him. If he nods at "hostage terms," do not explain further.

SLIDE 4 · The headline number · 4:30 to 7:00

SAY

"In 2024, Medicare Advantage insurers denied 4.1 million prior authorization requests. Only 11.5% of those denials were appealed. Of the appeals that were filed, 80.7% were partially or fully overturned.

Four in five appeals win. Almost nobody files one."

Stop talking.

THEN SAY (the caveat)

"That is Medicare Advantage, all services. It is not an imaging number, because CMS does not collect prior authorization data by type of service. It is the only audited, payer-reported, national figure that exists, and it describes the system your center bills into."

DEPTH / IF ASKED

Source: KFF, "Medicare Advantage Insurers Made Nearly 53 Million Prior Authorization Determinations in 2024," published 2026-01-28, analyzing data insurers are required to report to CMS. Mandatory reporting, not a survey.

Supporting figures from the same source: 52.8 million determinations in 2024, up from 49.8 million in 2023. 4.1 million denied, which is 7.7%, up from 6.4% in 2023 and 7.4% in 2022. The appeal rate was 11.7% in 2023 and 7.5% in 2019. The overturn rate was 81.7% in 2023. Over 90% of requests were fully favorable. Ninety-nine percent of Medicare Advantage enrollees are in plans that require prior authorization for some services.

What this number is not: a labor-savings number. It is an abandoned-revenue number. The binding constraint it reveals is staff capacity and follow-through, not clinical merit.

If he asks whether that money can be recovered: "That is why it leads. The denied-and-never-appealed volume is money the center is entitled to and never collects, for a reason that is about capacity, not merit. That is the ideal thing to automate: bounded, repetitive, deadline-driven, and abandoned under load. The recovery path is the lead build on slide 8. Detect, assemble, submit, track the clocks, and on denial draft the appeal for a human to sign. The appeal rate is the number to watch." Say the Medicare Advantage caveat before he asks, every time. Do not let this become a dollar figure. You cannot forecast recovered dollars without his payer mix and his denial volume, and he knows that. The honest framing is where the money is going and which number to watch, never what you will recover.

If he quotes an imaging denial rate of 25 to 40% at you: do not confirm it. Those are vendor-blog numbers with no methodology. Say, "I have seen that number and I could not trace it to a source I trust, so I will not use it." The peer-reviewed imaging figure is 4.94%, and it is coming at slide 10.

BEAT

Say "four in five," then stop. Count to five in your head if you have to. Let Fernando connect it. He will.

SLIDE 5 · Why greenfield matters · 7:00 to 8:30

SAY

"Forty percent of physicians report having staff who work exclusively on prior authorization.

You have not hired that person yet.

Two in five practices end up hiring someone whose entire job is prior auth. For you, the decision to automate or to staff is live right now. It will never be live again."

THEN SAY (the caveat)

"That is the AMA's 2025 survey of a thousand physicians. It is self-reported."

DEPTH / IF ASKED

Source: American Medical Association, 2025 AMA Prior Authorization Physician Survey. 1,000 practicing physicians, 400 primary care and 600 specialists, nationwide. Figures read directly from the report.

Frame it as revenue capture and capacity, never as headcount. Two reasons. First, El Paso wages run about 25% below the national average across all occupations: the El Paso metro all-occupation mean is $24.94 an hour against $33.54 nationally (BLS, May 2025 El Paso release 26-1364-DAL, dated 2026-08-13). A displaced-FTE argument is worth less here than it sounds. Second, Fernando came up as a technologist and has run departments. If he hears layoffs, he disengages politely and you never find out why. The line is: the work gets done without a bigger org chart.

Ask here: "How many prior authorizations a day were you running at steady state, and who did them?" His answer tells you whether he already lived this statistic.

BEAT

Pause after "you have not hired that person yet." That is the whole greenfield argument in one sentence.

SLIDE 6 · What an AI agent is · 8:30 to 10:00

SAY

"Before any more numbers, let me say what I mean by an agent, because the word gets used for everything.

There are two kinds. The first does a job inside your systems. Think of a very consistent employee who works one desk: a prior authorization, filling a cancelled slot from the waitlist, a records request. It works through your RIS and the payers' own electronic connections, not by clicking around a screen, and somebody on your staff approves anything that goes out.

The second one you talk to. You text it or call it: how many MRIs did we read last week, which referrer went quiet. It answers from your own numbers. And every Monday it sends you one short report: volumes by modality, reader, and referrer, authorization and denial rates, and the reconciliation.

Four guardrails, and they do not bend. A person signs anything clinical and anything that leaves the building. Numbers are calculated and checked, never guessed. Every vendor that touches patient data signs a HIPAA business associate agreement. And the data stays in the United States."

DEPTH / IF ASKED

"Calculated and checked" means the agent never does arithmetic in its head: totals, rates, and reconciliations are computed by code and checked, and if a number cannot be verified it says so instead of filling the gap. That is the answer to "how do I know it is right."

The owner's agent is read-only over operational data, inside the FDA administrative exclusion (slide 19). Do not promise a date for it. It is specified in the design and priced when they want it built.

Jesse is the one most likely to want the plumbing. Offer it once: "Jesse, happy to go through how it connects after lunch." HL7 v2 from the RIS, the payer's FHIR prior authorization API where it exists, EDI where it does not.

Never frame either kind as replacing anyone. It changes what your people do.

BEAT

Plain words, no acronyms on this slide. If Fernando nods through it, move on; if he asks a question, that question is the conversation.

SLIDE 7 · Part one: the thing you did not ask about · 10:00 to 10:30

SAY

"You asked Luis about scheduling, phones, and records. Those are real. The thing I would put first is the one you did not mention.

Prior authorization and eligibility verification. It is larger, and it has the best evidence."

DEPTH / IF ASKED

Nothing to defend. This is a transition.

BEAT

Plainly, without any edge. This is not a gotcha. Say it and advance.

SLIDE 8 · The problem: what prior auth costs you · 10:30 to 12:30

One slide, one idea (Rick, 2026-11-03): the hours slide and the 95% slide are now one slide, right after the Part one divider. One headline number, three lines, one closing line. Say it and let it sit.

SAY

"Physicians and their staff spend an average of 13 hours a week on prior authorization. About 40 requests per physician per week, each one worked by hand.

Three things about that work. Ninety-five percent of it is approved: at one institution, over four years, 17,913 outpatient MRI requests went in and 4.94% were denied. The labor goes into a process that nearly always says yes.

Nobody has time to appeal. Of the physicians who do not appeal denials, 52% say they do not have the staff time, and 59% say they do not expect to win. Those are exactly the two things an agent removes.

And CMS priced that same burden at 220 million provider hours over ten years, in the prior authorization rule itself.

A process that approves 95% of what it touches is one you automate, not one you staff."

THEN SAY (the caveats)

"The hours are survey data, self-reported, physician practices of all kinds, not imaging centers specifically. The 95% is one institution, one modality, four years; the national all-service denial rate from the KFF data was 7.7%. Both say

the same thing: most of the work is approvals."

"And the 220 million hours is CMS's projection in the rule. It is not measured savings. Nobody has gone back and counted yet."

DEPTH / IF ASKED

The hours and the requests: AMA 2025 Prior Authorization Physician Survey. Other figures from it if pressed: 32% say prior authorizations are often or always denied. 74% say denials increased over the past five years. 21% always appeal. 95% report care delays. 94% say it contributes to burnout. If he notices 13 hours here against the 14 hours CMS used: 13 is the 2025 AMA survey, 14 is the earlier AMA figure CMS cited in its 2024 rule. Say exactly that. The gap is a year of survey data, not a contradiction, and naming it unprompted is worth more than hiding it.

The 95%: Harrer S et al., J Am Acad Orthop Surg, 2025 Feb 15;33(4):e244-e252, PMID 39729599. Cooper University Health Care, orthopaedics, New Jersey payer mix, January 2019 through December 2022. 885 of 17,913 denied. Spine MRI was less likely to be authorized than extremity studies, and two Medicaid payers were significantly less likely to authorize. The authors' own conclusion: the low denial rate "underscores the questionable utility of the prior authorization process." This is the opposite of a scare statistic, and that is deliberate. Fernando has reconciled real remittances. Do not quote any inflated imaging denial rate; he will know.

The federal backstop: CMS-0057-F, 89 FR 8758, published 2024-02-08, Regulatory Impact Analysis, verified in the Federal Register full text. The dollar figure is at least 16 billion over ten years; lead with hours, because hours are the thing he actually paid for. Why it lands: the source is a binding federal rule, not a vendor and not a consultant, so it pre-empts "this is AI hype" before anyone says it. It is still a projection, which is why the caveat is mandatory. Give the caveat the same weight as the number.

Eligibility verification is the front half of the same workflow: it is where the "does this plan require authorization" determination happens. The EDI 270 and 271 eligibility transactions are the oldest and most standardized surface in this space, and the clearinghouse or billing vendor exposes them. There is no separate defensible benchmark on eligibility cost for imaging centers, so do not invent one.

Recommendation: build. This is the lead product. What the agent does: detects authorization-required orders at scheduling, assembles the documentation packet from the referral and the RIS, submits through the payer API where one exists, through EDI 278 where supported, and otherwise prepares a complete packet for a human to submit. Tracks every open request against the seven-day and 72-hour clocks. On denial, drafts the appeal with the clinical rationale and queues it for a human signature. Reports the appeal rate and overturn rate weekly, which is the number the owner should watch. The appeal rate is the headline of the Monday owner report on slide 16: it starts near zero for a new center by definition, it is measurable from week one, and it is entirely attributable to the system. A human signs every submission and every appeal in phase one.

The human signature is deliberate design, and it is the FDA boundary on slide 19, not a cost to be optimized away. If he asks "what if it makes something up": a person signs every payer submission and every appeal, and the system never recommends a study or judges whether one is appropriate. "Phase one" does not mean the sign-off quietly relaxes later. The specification states the graduation criterion: sign-off moves from every item to a sample only after a defined volume at a defined accuracy, measured, and only with the owner approving the change. Until then, every item. That turns the human step from a promise into a governed control, which is the language he negotiates in.

How the agent relates to the software, in one sentence if he asks: the software is the capability and the agent is the operator. Every piece gets built as a service with a clean interface that works without a model, and the agent runs it. The service is what goes in the specification and what any integrator can take over. The agent is replaceable. More on this at slide 16.

Why not a vendor: the category exists (Availity, Waystar, Cohere Health, Humata Health, Infinitus), but it is priced and implemented for health systems and large groups, and much of it is payer-funded. At his size, whether that is one center or three, he is the customer those vendors take last and configure least. And the appeal gap is not a technology gap. Nobody has time to file. That is a workflow problem a narrow agent and a human sign-off fix.

IN PLAIN WORDS (Rick, 2026-11-03), if he asks what "built for someone much larger" means:

"The prior authorization vendors out there are priced and implemented for hospital systems and large physician groups. A center your size is the customer they configure last and least. A lot of them are paid by the payers, not by you. And the real gap is not technology. It is that nobody has time to file the appeals. That is what I build for. If one of those vendors fits you, use it. I will tell you if it does."

Never disparage a vendor by name. "If it fits, use it" is the posture on every vendor question today.

If he says they are opening three centers, or asks whether any of this changes at three: "Three makes the design work more valuable, not less. The specification gets written once and deployed three times, and the four contract terms on slide 3 get negotiated once across all three. Three centers is a reason to buy the specification, not a reason not to." If he asks whether the price changes at three, the answer is under slide 25 (Rick, 2026-11-03: this center and one pick per system; same picks elsewhere is a smaller follow-on; different picks are quoted first). Do not assume a count. Use the number he gave you at the open, and if he did not give one, ask.

If he raises Olive AI: "Yes. Olive raised an enormous amount of money, shut down in 2023, and sold its prior authorization assets to Waystar and Humata Health. It sold broad automation to hospital systems on long implementation cycles. That is close to the opposite of one person building a narrow agent for one site he can watch directly."

BEAT

Say the headline number, then the three lines, then the last line, and let it sit for a beat. Slow down on "nobody has time." If short on time this slide is the headline number and the last line, nothing in between.

SLIDE 9 · The dates land on your opening · 12:30 to 14:30

SAY

"Since January first of this year, the payers under this rule have to decide a standard prior authorization in seven calendar days and an expedited one in 72 hours, and they have to publish their numbers.

On January first next year, they have to operate a FHIR Prior Authorization API, plus Patient Access, Provider Access, and Payer-to-Payer APIs.

Your target opening is about five months after that, with prior authorization already working and no legacy process to unwind. And almost 63% of medical plans say they are already building the prior authorization API ahead of the deadline."

THEN SAY (two caveats, both mandatory)

"The rule binds payers, not providers. It creates APIs my side can consume. It does not obligate your center to do anything, and it guarantees nothing about data quality or how easy the connection is."

"CMS-0057-F binds Medicare Advantage, Medicaid, CHIP and exchange plans on January 1, 2027. It does not reach employer-sponsored plans, TRICARE, or VA. So I am not selling you a compliance deadline. I am pointing out that the payers who move first set the standard the rest follow, and your build window lands exactly on that transition."

The second caveat is the demotion, and it is deliberate: the rule is the reason the window is now, not the reason to do this. Employer-sponsored coverage is the largest single block in El Paso County (268,142 people, 31.8% of the insured universe), and the rule does not reach it. If you let him find that, the regulatory claim breaks. If you say it first, it reads as command of the material.

IN PLAIN WORDS (Rick, 2026-11-03), say it after the two caveats if his face says "so what":

"Here is what the rule does and does not do. It says the payers have to have the connections. It does not say they will work well. Some payers will be late, some will be buggy, some will send incomplete data, and every one of them needs its own setup on my side. That setup is Phase 4 work, and I have planned for it. So the rule is the reason the timing is right. It is not the reason to buy."

IF THE FOUR-CHANNEL OBJECTION IS RAISED DIRECTLY ("most of our volume will be Fort Bliss and the VA")

"I looked that up, because you would know better than I would. In El Paso County, TRICARE-only and VA-only coverage together are under five percent of the insured population, about thirty-nine thousand people out of eight hundred forty-four thousand. Commercial and Medicare are where the volume and the margin are. The military channels matter for front-office workflow, not for the business case."

Exact figures if pressed: TRICARE-or-military-only 33,795 (4.0%), VA-only 5,037 (0.6%), combined 4.6% of 844,384 insured; employer-based plus direct-purchase 320,020 (37.9%). ACS 2024 5-year, table B27010, El Paso County. These are coverage shares of residents, never a share of his studies. Hand him the conversion: "Volume will skew older than coverage does; you would know the conversion better than I would." The number that matters more to his pro forma than either military channel: 21.6% uninsured, about 183,000 people. Self-pay policy and point-of-service collection are the real operational factor. Full plan-by-plan TRICARE and VA language is in the RUNBOOK under the Fort Bliss objection.

DEPTH / IF ASKED

Source for the dates: CMS-0057-F rule text. For Medicaid and CHIP managed care, the 2027 dates run from the rating period beginning on or after 2027-01-01; for exchange issuers, from plan years beginning on or after that date.

Source for the 63%: 2025 CAQH Index, FHIR and Interoperability module, published by DataSpring (formerly CAQH) on 2026-02-19. Payer self-report.

Commercial plans outside the rule: the design falls back to EDI 278 where supported, and otherwise a complete packet prepared for a human to submit.

A small signal that you read the rule and not a summary: drugs are excluded from CMS-0057, and CMS proposed to close that gap in April 2026. Irrelevant to imaging. Use it only if the conversation is already in the rule.

Ask here: "What is your payer mix going to look like?" Payer mix means what share of his studies will bill to each payer type: commercial insurers, Medicare Advantage, traditional Medicare, Medicaid and CHIP, exchange plans, and self-pay (the five groups are spelled out at slide 2). The rule reaches Medicare Advantage, Medicaid and CHIP, and the exchange plans. If he expects heavy commercial, say so out loud and put less weight on the January date. If he expects heavy Medicare Advantage and Medicaid, which is plausible in El Paso, the January date carries more weight. County coverage shares are verified as of 2026-10-12 (ACS 2024 5-year, B27010) and are in the RUNBOOK; quote them as coverage of residents, never as his volume. Medicare Advantage penetration in El Paso is NOT established and is the number that decides how much of his Medicare book the rule touches. Name it as a diligence item: "MA penetration is the number that decides how much of your Medicare book the new rule touches. I'd pull that before we price Phase 2." Do not guess it.

BEAT

Both caveats, every time. They are the two obvious counterattacks and you want to be the one who says them.

SLIDE 10 · One order through prior authorization · 14:30 to 17:30

SAY

"Let me make that concrete. An MRI order arrives at nine on a Tuesday.

It comes in from the referring office and lands in your RIS. The agent checks the patient's coverage and whether this payer requires authorization for this study. If it does, it builds the request from the order and the clinical notes and checks it against that payer's rules. Then it stops, and your staff member reviews it and approves. Only then does it go to the payer, electronically. The agent watches the payer's clock and puts the approval on the schedule the moment it lands. If it is denied, the agent drafts the appeal with the reasons and puts it in front of a person. Nothing goes out unsigned.

And you will not have to take my word for it. The demo exists today, on synthetic data, no real patients; you can click through it yourself. The production version is built against your real RIS and payers in Phase 4."

DEPTH / IF ASKED

The demo is live: https://rfinlay.github.io/imaging-priorauth-demo/ (Alan's build, synthetic data). It is on the slide and the leave-behind. Open it yourself Thursday morning; if it will not load, say "it exists, I will send you the link," never a date. Fernando is visual and distrustful after vendors over-promised; a demo he can click is worth more than any slide.

HOW TO RUN IT (switch to the demo here). Tab already open on the phone (cellular) and the laptop, opened before lunch, banner checked (green: live test server; orange: simulated payer, say so). Run **Maria** first: order, needs auth, packet, approve and submit, advance 3 days, approved. Then **Robert**: same steps fast, advance twice, denied, review appeal, sign and submit, advance, overturned. Then **Owner's Monday report**. Say the boundary line out loud before anyone asks: the test server stores the request but has no medical-necessity rules, so the approve or deny is simulated. A full page reload (Cmd+R, or pull-to-refresh on the phone) resets both orders to new; the "Start over" button does not. Reload before lunch and before handing the phone over. Full steps, lines and Q&A in `PRIORAUTH-DEMO-SCRIPT.md` (PriorAuth-Demo-Script.pdf).

THE LINK AND THE PACE (Rick, 2026-11-03). Thursday morning Rick texts the demo link to Luis; Luis forwards it to Fernando and Jesse. At the table they scan the QR on slide 10 (or the leave-behind) or open the text, and reload once for a fresh start. Rick leads the pace on split screen, laptop beside the deck, and says each step before anyone clicks; everyone waits for him on **Advance 3 days**, because clicking it early puts their copy ahead of the room. Pricing is not on this slide; it comes on slide 25, after the close.

If he raises Exa building something similar. Never disparage Exa.

> "They may well, and if theirs is good, use it. What I build is vendor-neutral: it runs on your data and your contract terms, and it works with whatever RIS and PACS you pick, Exa included. The four access terms protect you either way, whether the feature comes from them or from me."

Then stop. The four terms (slide 3) are the real answer: they make the center's data reachable whoever builds the agent.

If he brings his own idea, prior-record retrieval for one: give him the floor and write it down. It is the records build (slide 15). Luis says outside priors come by courier on CD or DVD and after five days the study is read without them; that is the problem in his own words.

Plumbing for Jesse: HL7 v2 orders out of the RIS; the payer FHIR prior authorization API where required (Medicare Advantage, Medicaid, CHIP and exchange plans from 2027-01-01); EDI 278 or a human step where not. No screen-

driving of payer portals; the AI vendor's BAA does not cover it (slide 20).

BEAT

Step four is the safety model. Pause on it. Then let Fernando ask "when can I see it."

SLIDE 11 · What a prior auth process actually does all day · 17:30 to 18:00

SAY

"Here is the other side of it. At one institution, over four years, 17,913 outpatient MRI prior authorization requests went in. 4.94% were denied. The other 95% were approved.

A process that approves 95% of what it touches is one you automate, not one you staff."

THEN SAY (the caveat)

"That is one institution, one modality, four years. The national all-service denial rate from the KFF data was 7.7%. Both say the same thing: most of the work is approvals."

DEPTH / IF ASKED

Source: Harrer S et al., J Am Acad Orthop Surg, 2025 Feb 15;33(4):e244-e252, PMID 39729599. Cooper University Health Care, orthopaedics, New Jersey payer mix, January 2019 through December 2022. 885 of 17,913 denied. Spine MRI was less likely to be authorized than extremity studies, and two Medicaid payers were significantly less likely to authorize. The authors' own conclusion: the low denial rate "underscores the questionable utility of the prior authorization process."

This is the opposite of a scare statistic, and that is deliberate. Fernando has reconciled real remittances. Do not quote any inflated imaging denial rate; he will know.

Recommendation: build. This is the lead product. What the agent does: detects authorization-required orders at scheduling, assembles the documentation packet from the referral and the RIS, submits through the payer API where one exists, through EDI 278 where supported, and otherwise prepares a complete packet for a human to submit. Tracks every open request against the seven-day and 72-hour clocks. On denial, drafts the appeal with the clinical rationale and queues it for a human signature. Reports the appeal rate and overturn rate weekly, which is the number the owner should watch. The appeal rate is the headline of the Monday owner report on slide 17: it starts near zero for a new center by definition, it is measurable from week one, and it is entirely attributable to the system. A human signs every submission and every appeal in phase one.

The human signature is deliberate design, and it is the FDA boundary on slide 21, not a cost to be optimized away. If he asks "what if it makes something up": a person signs every payer submission and every appeal, and the system never recommends a study or judges whether one is appropriate. "Phase one" does not mean the sign-off quietly relaxes later. The specification states the graduation criterion: sign-off moves from every item to a sample only after a defined volume at a defined accuracy, measured, and only with the owner approving the change. Until then, every item. That turns the human step from a promise into a governed control, which is the language he negotiates in.

How the agent relates to the software, in one sentence if he asks: the software is the capability and the agent is the operator. Every piece gets built as a service with a clean interface that works without a model, and the agent runs it. The service is what goes in the specification and what any integrator can take over. The agent is replaceable. More on this at slide 17.

Why not a vendor: the category exists (Availity, Waystar, Cohere Health, Humata Health, Infinitus), but it is priced and implemented for health systems and large groups, and much of it is payer-funded. At his size, whether that is one

center or three, he is the customer those vendors take last and configure least. And the appeal gap is not a technology gap. Nobody has time to file. That is a workflow problem a narrow agent and a human sign-off fix.

IN PLAIN WORDS (Rick, 2026-11-03), if he asks what "built for someone much larger" means:

"The prior authorization vendors out there are priced and implemented for hospital systems and large physician groups. A center your size is the customer they configure last and least. A lot of them are paid by the payers, not by you. And the real gap is not technology. It is that nobody has time to file the appeals. That is what I build for. If one of those vendors fits you, use it. I will tell you if it does."

Never disparage a vendor by name. "If it fits, use it" is the posture on every vendor question today.

If he says they are opening three centers, or asks whether any of this changes at three: "Three makes the design work more valuable, not less. The specification gets written once and deployed three times, and the four contract terms on slide 3 get negotiated once across all three. Three centers is a reason to buy the specification, not a reason not to." If he asks whether the price changes at three, the answer is under slide 29 (Rick, 2026-11-03: this center and one pick per system; same picks elsewhere is a smaller follow-on; different picks are quoted first). Do not assume a count. Use the number he gave you at the open, and if he did not give one, ask.

If he raises Olive AI: "Yes. Olive raised an enormous amount of money, shut down in 2023, and sold its prior authorization assets to Waystar and Humata Health. It sold broad automation to hospital systems on long implementation cycles. That is close to the opposite of one person building a narrow agent for one site he can watch directly."

BEAT

Land the last sentence and let it sit for a beat before you move to Part two.

SLIDE 12 · Scheduling: the empty slot is not a no-show · 18:00 to 19:30

SAY

"A University of California, Irvine study of about nineteen thousand outpatient radiology exams found 22.3% were cancelled and 2.0% were no-shows. Seventy percent of the cancellations were patient-initiated. Most empty slots are known in advance. That is a backfill problem, not a reminder problem.

Nearly everyone pitching imaging automation pitches no-show reduction. That is the 2% number. The cancellation stream is ten times larger, and most of it arrives with lead time.

What I would build here is small. A waitlist that sees a cancelled slot and works a ranked list of patients who want to come in sooner. It sits on top of whatever RIS you choose."

THEN SAY (the caveat)

"That is one academic center over 2022 and early 2023. Your numbers will differ. The pattern will not: cancellations outnumber no-shows many times over, and most of them come with notice."

DEPTH / IF ASKED

Source: Aijaz A et al., "Sociodemographic Factors Associated with Outpatient Radiology No-shows Versus Cancellations," Acad Radiol, 2024 Aug;31(8):3406-3414, PMID 38705764. UC Irvine. 19,262 exams, 9,713 patients. 24.3% missed in total: 22.3% cancellations, 2.0% no-shows. 70.19% of cancellations patient-initiated. The same study found uninsured and publicly insured patients 1.3 to 4.9 points more likely to miss, and residence in a disadvantaged neighborhood added 4.9 points. Both matter in El Paso. Single tertiary academic center, so high confidence as a finding and medium as a benchmark.

Pocket figure: across 2,893,626 outpatient imaging visits over sixteen years, booking more than six months out carried an odds ratio of 3.18 for no-show versus booking within a week. Shortening time-to-appointment reduces no-shows on its own, and backfill is how you shorten it. (Rosenbaum JI et al., J Am Coll Radiol, 2018 Jul;15(7):944-950, PMID 29755001.) That institution's overall no-show rate was between 2.26% and 3.36%; mammography was highest at 6.99%, radiography lowest at 1.25%.

Do not quote general no-show benchmarks of 10 to 20%, "no-shows cost a practice a million a year," or "$200 per unfilled slot." All of those are vendor material with no methodology, and they contradict the peer-reviewed rates.

The one real risk in this build is TCPA, the Telephone Consumer Protection Act, the federal law that limits automated calls and texts to patients: a backfill offer counts against the same three-per-week cap as reminders. That is the rate governor on slide 20.

The cancellation question is the lead-in on slide 10, asked before this slide comes up.

BEAT

If his number is nowhere near 22%, say so and keep the pattern, not the figure: cancellations outnumber no-shows, and most arrive with notice.

SLIDE 13 · Scheduling: what reminders honestly do · 19:30 to 20:30

SAY

"A randomized trial at Massachusetts General, about seven thousand MRI patients, added a text reminder on top of the phone call they were already making. No-shows went from 5.1% to 3.8%. It did not improve arrival punctuality."

THEN SAY (the caveat)

"That is 1.3 percentage points absolute. Vendors will tell you that is a 25% reduction. Same study. And only about two thirds of the texting group could actually be reached by text.

Your RIS will do this. Let it."

DEPTH / IF ASKED

Source: Liu C et al., "Text Message Reminders Reduce Outpatient Radiology No-Shows But Do Not Improve Arrival Punctuality," J Am Coll Radiol, 2017 Aug;14(8):1049-1054, PMID 28583321. Massachusetts General Hospital. n = 6,989, randomized by day, p = 0.02, odds ratio 0.75 with a 95% confidence interval of 0.59 to 0.94. Only 67.5% of the texting arm was reachable by text. 2016 data, which is its main weakness.

It measures the incremental effect of texting on top of phone calls already being made, which is the real decision a center faces.

There is a NEJM Catalyst study reporting that adding an automated voice call to texts lowered no-shows a further 1.7 points (9.6% versus 11.3%). The article is paywalled and was not read; the figures came through a search summary. Do not lead with it.

Recommendation: buy, meaning the RIS. Self-scheduling, reminder sequences, and intake forms are bundled into modern cloud imaging RIS products.

If he says "doesn't our RIS already do this?": "For scheduling, reminders, self-scheduling, and intake forms: yes, and it should. I am telling you to buy that and use it. RamSoft's imaging-center product, for one, advertises self-scheduling, automated notifications, and a workflow designer. What I have not seen any RIS in this tier claim is eligibility verification or prior authorization. That gap is where the strongest evidence is, and it is the piece you did not ask

about." Hold that line exactly: you have not seen it advertised. Do not claim the RIS definitely lacks a prior auth module. The specification verifies it against whichever RIS they pick.

BEAT

Quote the absolute numbers and let him do the arithmetic. An exited founder will smell the relative-risk trick, so be the one who names it.

LEAD-IN TO SLIDE 12 (say before you advance)

"Who takes the phone call when a patient describes chest pain?" His answer makes the next recommendation obvious before you state it.

SLIDE 14 · Answering patients: buy it, do not build it · 20:30 to 22:00

SAY

"Phones are a real cost. You are right about that. My recommendation is to buy a healthcare voice vendor, not to have me build one. I will write the specification, run the evaluation, integrate it, and supervise it. And I will tell you when it is not good enough in Spanish.

Here is why. 26.5% of El Paso County residents over five speak English less than very well, and 97.8% of them speak Spanish. One in four of your patients cannot be served by an English-first system.

A voice agent that mishandles a patient describing chest pain, or garbles Spanish, is a clinical safety event and a reputational event. It is not a bug. Voice is the highest blast-radius failure on this whole list, and it is a funded category I cannot beat on cost.

I would rather tell you to buy something than build something that is not mine to build."

DEPTH / IF ASKED

Source: American Community Survey 2024 five-year, Table B16004, El Paso County. Population five and over of 813,194. 215,186 speak English less than "very well," which is 26.5%. 537,316 speak Spanish at home, which is 66.1%. Computed from raw ACS counts via Census Reporter, because census.gov blocked automated access. Say 26.5%, not 66%: the operational number is the limited-English number, and knowing the difference shows you understand a requirement versus a demographic.

Vendor category names if asked: Hyro, Assort Health, Infinitus, Parlance, Syllable. None of them publish pricing. Do not quote any.

TCPA: an AI voice is an "artificial voice" under the FCC's February 2024 ruling (FCC 24-17), and that ruling expressly preserved the healthcare exemption. Inbound calls the patient initiates are outside the outbound autodialer and artificial-voice restrictions. Any outbound callback from the voice system falls back under them. Texas HB 149, effective 2026-01-01, extends an AI disclosure obligation to healthcare providers; scope not fully verified, so plan a disclosure at the start of the call.

The honest alternative: do not do voice at all in phase one. Let the front desk answer the phone while the prior auth agent takes that load off them.

What integration needs: a RIS scheduling API for the vendor to book against (a contract term from slide 3), a vendor BAA, an escalation path to a human on any symptom language, and a Spanish evaluation set built with Luis.

BEAT

Say the buy line plainly. Recommending a purchase here makes everything you do propose to build more believable.

SLIDE 15 · Records: the clock and the fee rule both favor automation · 22:00 to 23:30

SAY

"Records requests. Two rules work in your favor here, and one problem works against you.

The first rule. The 30-day HIPAA clock runs from the day the request is received, not from the day it reaches the right desk. One extension of up to 30 more days, with written notice inside the first 30. Outer limit, 60 days. A request sitting in a pile is already burning the clock.

The second rule. The fee you can charge covers the labor to copy, supplies, postage, and a summary if the patient agreed to one. Search and retrieval labor is explicitly not chargeable. Finding the study is the expensive half of the work, and you cannot bill for it.

The problem. The harder side is outbound. A new center opens with zero priors on file. Every comparison study comes from somewhere else. RSNA is on record this year that DICOM exchange is still not a TEFCA requirement. Patients still carry their own discs. Nobody has solved that for a small center, and I am not going to tell you I have. The part I can build is the tracker: which facility accepts which kind of request, generate it, track it, and import what comes back."

THEN SAY (the caveat)

"Texas has its own records deadline that is shorter than the federal 30 days: fifteen business days from a written request for an electronic record. That is the clock the tracker is built to, and it is verified against the statute in the specification."

(Fifteen business days, Texas Health and Safety Code 181.102, verified by Sherlock 2026-11-03 against the official text. It applies to a provider using an electronic health records system that can fulfill the request. The full chapter 181 depth is under slide 20.)

DEPTH / IF ASKED

The clock and the fee components: 45 CFR 164.524. The dividing line for billable labor, from the 2016 HHS guidance as quoted in Ciox: copying labor begins only once the responsive PHI "has been identified, retrieved or collected, compiled and/or collated, and is ready to be copied."

Ciox Health v. Azar, 435 F. Supp. 3d 30 (D.D.C. 2020), vacated the third-party-directive expansion but did not vacate the $6.50 flat-fee option. The court held the listing of fee methods was not reviewable agency action and never reached the merits. The option remains.

OCR's Right of Access Initiative has produced more than 50 enforcement actions. "More than 50" is supportable. Do not quote a settlement figure.

RSNA comment letter to ASTP/ONC, March 2026: names the prior-study retrieval gap directly, says patients still act as couriers for their own imaging, flags it for small community centers, and asks that DICOM exchange become a TEFCA requirement.

Recommendation: build, selectively. Inbound intake, deadline tracking, fee calculation, and status reporting are a small custom build with clear regulatory specifications and hard deadlines. Deadline tracking is the first feature, not the last, because the failure mode is a missed statutory deadline and an OCR complaint. Patient identity verification before release stays a human step in phase one. The retrieval itself is a DICOM query against the PACS, which is why the DICOM permission on slide 3 matters.

The local facility directory gets built by hand in the first months: the hospital systems and the independent groups. Do not name Southwest X-Ray or Akumin when you talk about where priors will come from.

Do not quote a cost per records request. The circulating "$50 to $200 per request" is a vendor figure and unverified.

BEAT

Do not say "from running two centers" or "two companies." He built one company with five centers. Let the fee rule land; it is the one most owners have never heard stated.

LEAD-IN TO SLIDE 14 (say before you advance)

"What do you want to see on one page every Monday morning?" Whatever he says is the specification. Write it down before the slide comes up.

SLIDE 16 · The fifth candidate: the numbers the owner actually looks at · 23:30 to 24:30

Cut this slide first if you are running long at 23:30. The Monday-morning question is the lead-in on slide 15; if you did not get to it there, ask it on this slide.

SAY

"The fifth one is the numbers the owner actually looks at. Studies by modality, by reader, by referrer. Turnaround. Scheduled versus completed. Revenue per slot.

All of it already lives in the RIS and the billing system. The problem is that those are two systems, and RIS reporting is built for radiology operations, not for an owner.

I have no study to show you here. I have the fact that you will want a referrer-and-revenue view, and the RIS will not give it to you. It is the smallest compliance footprint of the five, and the cheapest to build.

Here is how I would write it down. As an owner, I want to ask one question on a Monday morning, which referrer went quiet, what is the appeal rate, are we being paid for what we read, and get the answer from my own numbers, so that I run the center on facts and not on a report someone has to build for me. Your manager and your front desk lead want the same numbers from their side: today's schedule, the open authorizations, the slots that just opened.

And the version I would build is one you can ask questions of. Not a dashboard you read. Something you ask, in plain words: why were we down last Tuesday, what is the appeal rate this month, which referrer went quiet. It reads your numbers and answers. It never touches a patient."

DEPTH / IF ASKED

There is no external evidence for this candidate, and the research says so plainly. No benchmark literature exists on what imaging center owners want from dashboards. Say "I have no study" rather than reaching for one.

The structural claim holds without a study: the owner's view requires joining RIS data to billing data, and those are usually different systems. Both exports are contract terms from slide 3.

Custom by nature, because it encodes one owner's definition of what matters. The reader-reconciliation view already has a customer in Luis. FDA: "business analytics" and "practice and inventory management" are named in the statute at slide 19. PHI is present, so the BAA and Texas residency still apply, but there is no patient-facing surface.

The owner agent, and how every agent in this design is built. Three kinds of agent, by how much damage a mistake can do.

- **The owner agent.** Read-only conversational access over operational data: volumes, cancellations, no-shows, payer mix, appeal rate, reader turnaround. He asks a question in plain words and gets an answer from his own numbers. Lowest risk on the whole list: it reads and never acts, it sees nothing the owner does not already see, it has no patient contact, and it has no clinical surface. It is "business analytics" and "practice and inventory

management," named in the statute at slide 19. It is specified in Phase 2 and it is the strongest candidate for the first thing he sees working. It reads your own database, so the AI vendor's BAA exclusions on slide 20 do not touch it.

- **The workflow agents.** Prior auth, records, backfill. They act on the world, and a human signs. Already specified, bounded, medium risk. No change.
- **Patient-facing voice.** Highest blast radius. Still bought from a vendor. No change.

The design standard that applies to all three, and say it if he asks how any of this is built: the software is the capability and the agent is the operator. Every automation is two deliverables. First, a deterministic service with a clean interface that works without any model at all. Second, an agent surface over it. The service is auditable, testable, and survives any model change; it is what goes in the specification and what any integrator can take over, which is what keeps the slide 21 continuity promise honest. The agent is the replaceable part: the model vendor can change, the BAA envelope can change, the pricing can change, and none of it breaks the center. That is also the honest answer to "can an AI do this": the agent is a better interface, the service is the capability.

Scope effect, so nobody misreads it: the owner-agent specification and the tool-first standard are inside Phase 3 (design). The price does not move. See slide 25.

BEAT

Write down his Monday-morning answer. That sentence is worth more than the slide. If he lights up at any of it, treat it as a requirement, not a reaction: ask what he would want it to answer first, and write his exact words down. The first question he names is the first report the system has to answer.

SLIDE 17 · The scorecard · 24:30 to 25:30

SAY, in one breath

"Prior authorization and eligibility: build. Strongest evidence, the vendors are built for someone much larger than you, and the federal APIs arrive January first.

Reminders and self-scheduling: buy. That is the RIS.

Cancellation backfill: build, small, on top of the RIS. That addresses the 22%, not the 2%.

Voice: buy. I integrate.

Records coming in, the requests: build, selectively.

Records going out, the priors: build, selectively. That is the tracker.

Owner reporting: build.

Billing, claims, and the eligibility feed: buy. That is a clearinghouse function with a real vendor market, and I am not touching your claims work.

Three of the eight, I am telling you to buy."

DEPTH / IF ASKED

The slide says what each piece does; the why is here (Rick, 2026-11-03). Prior authorization: strongest evidence, the vendors are built for someone much larger, the federal APIs arrive January first. Reminders and self-scheduling: commodity, bundled in modern imaging RIS products. Backfill: a ranked waitlist on top of the RIS that addresses the 22.3% of cancellations, not the 2.0% of no-shows. Voice: a funded category with the highest blast radius and an unforgiving Spanish bar. Records, inbound requests: hard statutory deadlines (fifteen business days in Texas) and

retrieval labor you cannot bill for; the build is intake, an identity check step, the deadline clock, the fee calculation, status, and a DICOM export of the study. Records, outbound priors: no vendor has solved it for a small center; the build is a request tracker to other facilities, follow-up, import on arrival, and a flag to the radiologist when a prior is still missing. Owner reporting: custom by nature, smallest compliance footprint. Billing: a commodity clearinghouse function with a real vendor market.

Every row except billing was defended on its own slide. **Billing is the new row and it is here for a reason: Phase 2 evaluates a billing vendor, and before this row existed there was no slide that justified it.** Fernando has reconciled real remittances for twenty years, so billing is the area he knows best and this is the row he is most likely to stop you on.

The answer, and it is a short one:

> "Billing is a buy and I am not touching the claims work. What I evaluate is whether the billing vendor's eligibility and claim data are reachable, because the prior authorization system I want to build depends on them, and whether the four access terms go into that contract the same way they go into the RIS contract."

That is the whole answer. The prior auth agent in row 1 cannot function without eligibility and remittance data, so the billing contract is an access question, not an accounting question. Do not volunteer an opinion about any specific billing vendor; you have not evaluated one, and that evaluation is what Phase 2 is for.

⚠️ **The one seam on this slide that can look self-contradictory, so know it cold.** Row 1 is "prior authorization **and eligibility**: build." Row 8 is "billing **and the eligibility feed**: buy." A careful listener hears eligibility twice, on both sides of the line. Fernando is a careful listener. The distinction is real and it is one sentence:

> "I am not building an eligibility system. Your billing vendor already has that transaction; the EDI 270/271 eligibility check is the oldest standardized surface in this business and every clearinghouse exposes it. What I build is the thing that reads it and acts on it, which is the part nobody sells you. That is also why the access terms in that billing contract matter to me."

Memorize that. If you fumble this one, the slide stops being a credibility move and becomes a thing he has to untangle for you.

IF ASKED "Which RIS and PACS should we buy?" (Rick, 2026-11-03). The position for a single center is one cloud system that bundles RIS and PACS. Say it in four sentences:

> "For a center your size I'd lean toward one cloud system for RIS and PACS: one vendor, fewer interfaces, faster opening. The contract is what keeps you free to leave. Picking the vendor is Phase 2."

Why one system: one vendor, one support line, one contract; no RIS-to-PACS interface to pay for or keep alive; one worklist; faster go-live; the lean control Fernando wants. The cost is lock-in and giving up best-of-breed, and the four contract terms on slide 3 are the mitigation: data export in DICOM and HL7 at no penalty, interface fees fixed at signature, API access, worklist access. Split RIS from PACS only if the radiologists have a strong PACS preference or want multi-site reading from day one. Dictation is selected separately, and the radiologists choose it. Exa is one example of an integrated RIS, PACS and billing system, never the pick; the starting shortlist is slide 30 in the appendix, and no vendor is named as the choice until Phase 2.

BEAT

Read it in one breath. **"Three of the eight, I am telling you to buy" is the point of the slide**, because recommending against yourself three times in a row is the most credible thing you say today. Let it sit before you

move.

SLIDE 18 · Proof: RadAssist, in daily use · 25:30 to 26:30

Updated 2026-11-03: overview only, no live demo (Luis's feedback, relayed by Rick). One minute on the slide, spoken, then the table goes to Luis. The three-minute Spine MRI walkthrough is kept in archive/DEMO-SCRIPT.md Part A, marked not used Thursday, in case Luis himself asks you to open it; nothing on this slide depends on a signed-in tab, a hotspot, or the case card.

SAY (about one minute)

"Enough slides for a minute. This is RadAssist. Structured radiology reporting, in production, and Luis uses it every day on real patients.

Four modules today: calcium scoring, thyroid ultrasound, spine MRI, and CT spine. It runs on his own templates and his own rules: one dropdown per sentence, and the impression writes itself in the order he set. It drafts. He reads it, and he signs.

Nothing leaves the browser. No patient data sits on a server; the finished report is plain text he pastes into whatever system the center buys. It does not read an image, so it is not a device, and it will not read one until there is a regulatory path for that.

The spine module is the one I would point to, because of how it got built. Luis sent me his template and his rules. Every rule in it is his."

Then hand the table to Luis. "Luis, tell him about a Tuesday. What broke, and how fast did it get fixed? What did you ask for that got built?" Stop talking.

DEPTH / IF ASKED

For Fernando, every sentence of this is control and simplicity: his template, his rules, his signature; the report is plain text he pastes into whatever system the center buys; nothing is sent anywhere. No vendor lock, no integration before it is useful. Never "scale," never "platform."

If Luis mentions something that broke, let him. The speed of the fix is the proof, not the absence of failures.

If Fernando asks to see it: let Luis answer first; it is his tool and his call. If Luis wants it shown, the walkthrough in archive/DEMO-SCRIPT.md Part A is ready, but it needs a signed-in tab, which you will not have unless you set it up. The honest line: "Luis can show you on a working day; today I would rather you hear him describe it than watch me click."

If Fernando connects this to the FDA slide coming up: the same discipline that keeps the agents administrative is what keeps RadAssist on the right side of the line. It formats what the radiologist graded; it does not read the image. Keep that to one sentence.

Roadmap, as Luis asked for it (on the slide): voice dictation integrated into RadAssist, then brain and cardiac CTA (CAD-RADS) modules. Luis will email guideline-display corrections; treat them as his. Say "next on the roadmap," never a date. He also wants technologists capturing data at the ultrasound station so reports pre-fill; note it, do not promise it. **AI image auto-fill** (Luis's vision from the 10-31 call: upload the per-level images and have the software fill the level findings) is diagnostic software. On the slide and out loud it is "a future item with a regulatory path," never a promise, never a date.

Selling RadAssist to other centers: speaking-notes only, never in front of Fernando. Luis sees resale potential to competitor centers for structured reporting. It is real and worth a private conversation with Luis. At this table it collides with slide 22, where you tell Fernando this center is the priority and his service terms do not change if you

take on a second site. Both are true; only one belongs in this room. Nothing about how Luis came to use RadAssist, or on what terms, is said at this table (Rick, 2026-11-03).

BEAT

One minute, no more. Hand the table to Luis. Do not fill his pauses.

THE ONE LINE ON THE RIS PATH (Rick, 2026-11-03; the report-assistance slide is now appendix slide 35)

Say it once, from the last line on the slide: "At your center the signed report flows into the RIS over HL7 version 2, a contract term from slide 3. I verify the inbound path against the RIS you pick before you sign."

If records came up and you want the direction settled, ask it here, to Luis as much as to Fernando: "When your partner said requesting medical records, did he mean the report you write going out, or the priors and outside records coming in?" Write down which. The two-directions answer, the named DICOM path and the across-centers deployment default are all on slide 35, only if asked.

LEAD-IN TO SLIDE 17 (say before you advance)

"Fernando, the question you are about to ask is whether any of this is a medical device. The answer is in the statute, not from me."

SLIDE 19 · Boundaries: the FDA question, answered by the statute · 26:30 to 27:30

SAY

"The FDA question, answered by the statute and not by me. Federal law says software for administrative support of a health care facility is not a medical device, and it lists what that means by name. Claims or billing information. Appointment schedules. Business analytics. Determination of health benefit eligibility.

Every candidate on that scorecard is on the list by name.

And the agents stop at the clinical boundary. Nothing interprets an image, selects a protocol, or recommends a study."

DEPTH / IF ASKED

Source: 21 U.S.C. 360j(o)(1)(A), read verbatim at Cornell Legal Information Institute. The full list also includes financial records, information about patient populations, admissions, practice and inventory management, and laboratory workflow. Section 360j(o)(2) adds that FDA "shall not regulate" excluded administrative functions even when they coexist with device functions in one product.

Lead with the list, not with any engineering rule about orchestration. The list contains the words "appointment schedules" and "determination of health benefit eligibility." That is a complete answer and it invites no debate about how independently a clinician reviews anything.

The one real boundary: the moment an agent touches image data interpretively, or recommends which study to order, it leaves this exclusion and enters device territory under the clinical decision support criteria at 360j(o)(1)(E), whose guidance was revised 2026-01-06 and 2026-01-29. Protocol selection and clinical appropriateness are the live hazards in imaging. Say explicitly that the agents stop there.

Texas SB 1188 separately requires disclosure when AI is used for diagnostic purposes. None of these five are diagnostic. Another reason to keep the line clean.

The owner agent on slide 16 sits inside this exclusion, not near the edge of it. It reads operational and financial data and answers questions about them, which is "business analytics" and "practice and inventory management" by name.

It does not read images, select protocols, recommend studies, or touch a patient. If he asks whether "an agent that can tell me anything about my business" crosses the line: anything about the business, nothing about a patient's care. The line does not move to make room for it.

If "won't regulators come for this" comes up: "The state that has legislated hardest on AI patient communication, California, expressly excludes appointment scheduling and billing from its disclosure law." (AB 3030, subsection (c)(7).)

BEAT

One minute. The list is the answer. Do not argue it.

SLIDE 20 · Boundaries: what I will design around, stated before you ask · 27:30 to 29:00

SAY

"Four constraints I am going to design around, stated before you ask.

Texas SB 1188. Electronic health records with Texas patient data have to be physically stored in the United States as of January first this year. Every component, including the model provider, gets verified for US residency before anything is promised.

TCPA, the Telephone Consumer Protection Act. That is the federal law on automated calls and texts. Its healthcare exemption allows one message per day and three per week per patient. No billing content. Provider name and opt-out in every message. Five hundred dollars per violation, up to fifteen hundred if willful. A reminder plus a backfill offer plus a results notice can collide with the three-per-week cap. The system needs a per-patient rate governor, not just a send queue. I've built per-user limits like that in my own production systems, and this one gets designed in from day one.

The AI vendor's business associate agreement. Anthropic's agreement now covers Skills, Web Search, Memory, and the Bash and Text Editor tools. It still excludes Computer Use and Web Fetch, which are exactly what you would reach for to drive a payer portal that has no API. So the prior auth design routes through APIs, EDI, or a human step. Not screen-driving.

Texas HB 300. Texas privacy law covers me directly, not just through our contract. If I mishandle your patients' data, I'm the one the state comes after. That's how it should be."

THEN SAY (the caveat, on the BAA)

"The coverage depends on which version of the agreement is signed and when. I will confirm the exact envelope in writing before anything with patient data is built."

TALK IT THROUGH, one block per boundary (Rick, 2026-11-03). Plain English, for when a line lands and he wants it explained. What it is, what it means for the center, the one line to say.

- **SB 1188.** *What it is:* a Texas law that says electronic health records holding Texas patients' data must be physically stored in the United States. *What it means for the center:* every system and every vendor, including the AI company, has to show in writing where the data sits; a cloud account locked to a US region satisfies it. *The one line:* "Every component gets verified for US residency before anything is promised."
- **TCPA.** *What it is:* the federal law on automated calls and texts; its healthcare exemption lets a provider send appointment-type messages without separate marketing consent, within limits. *What it means for the center:* one message a day and three a week per patient across every message type, nothing about money, the provider's name and an opt-out in every message. The "per-patient rate governor" is a counter: for each patient it counts every text and call the systems send, from reminders to backfill offers to results notices, caps them at one a day and three a

week, blocks anything with billing content, and stamps the name and the opt-out on each one. *The one line:* "I've built per-user limits like that in my own production systems, and this one gets designed in from day one."

- **The AI vendor's BAA.** *What it is:* the HIPAA business associate agreement the AI company signs, which makes it legally responsible for patient data it receives. *What it means for the center:* patient details do go to the AI company, the same way they go to the clearinghouse, under that signed agreement, never kept and never trained on; the agreement does not cover the browser-driving tools, so nothing drives a payer portal screen with patient data. *The one line:* "Under a signed HIPAA agreement, never kept, never trained on, and never through a screen the agreement does not cover."
- **HB 300.** *What it is:* Texas's own health privacy law, broader than HIPAA. *What it means for the center:* training records, a fifteen-business-day records deadline, no sale of patient data, and the Attorney General as the enforcer; and it reaches Rick personally, not only through the contract. *The one line:* "If I mishandle your patients' data, I'm the one the state comes after. That's how it should be."

DEPTH / IF ASKED

SB 1188, 89th Legislature, signed 2025-06-20, effective 2025-09-01, with the storage requirement effective 2026-01-01 and applying to all records stored on or after that date. Passage and dates verified at capitol.texas.gov. The provision wording comes from six or more converging law-firm analyses, not the enrolled text, so if pressed on exact wording say "as I read it."

TCPA: 47 CFR 64.1200(a)(9)(iv), the wireless healthcare exemption, all eight conditions: sent only to the number the patient provided; provider name and contact at the start; content limited to appointment confirmations and reminders, pre-registration instructions, lab results, and the other enumerated categories; no telemarketing and no accounting, billing, debt-collection, or other financial content; one minute or less for voice and 160 characters or less for text; one per day and three per week per patient, voice and text combined; opt-out in every message; opt-outs honored immediately. Damages under 47 U.S.C. 227: $500 per violation, trebled to up to $1,500 if willful or knowing. $1,500 is the ceiling, not a fixed amount. Balance-due and payment reminders cannot ride this exemption at all; they need their own consent basis, which is why they are declined for phase one.

BAA: Anthropic's BAA documentation at privacy.claude.com, accessed 2026-10-01. Also excluded: Files API, Skills API, Code Execution, Batch API, and third-party data flows through external MCP connectors. Enterprise coverage applies to agreements accepted after 2025-12-02; Messages API feature coverage after 2026-04-01. If he asks "can your AI vendor even handle patient data": "It covers the parts of the architecture I would use. It does not cover Computer Use or Web Fetch, so I will not promise screen-driving a portal with patient data, because I cannot do it inside the agreement today."

HB 300 is Texas Health and Safety Code Chapter 181, and it is the reason the fourth constraint reaches you personally. What the chapter covers, so you can go one level deeper than the slide:

- **Who it reaches.** Its definition of a covered entity is broader than HIPAA's: anyone who assembles, collects, stores, uses, or transmits protected health information about a Texas patient, in any form. That reaches you directly, not only through the center's business associate agreement. Say: "I carry that obligation in my own right, whether or not a BAA says so."
- **Training.** Every employee who touches patient data must be trained on state and federal privacy law within **90 days of hire**, and sign a statement of completion, which the entity keeps for **six years**. Retraining is required only after a **material change in the law**, within a year of it taking effect. There is no annual or two-year refresher in the statute: the original 2011 bill had one and the Legislature removed it in 2013. **Never say "every two years."** For you that means your own training record and the center's.
- **Electronic records on request.** A provider whose electronic records system can fulfill it must give a patient their electronic record within **15 business days of a written request**, in electronic form unless the patient agrees to

another. HIPAA allows 30 calendar days. That is the records clock on slide 15, and it is the one the tracker is built to.

- **No sale of PHI.** Protected health information cannot be disclosed for remuneration. The exceptions are treatment, payment, health care operations and insurance functions with another covered entity, or where another law requires it; none of them covers anything in this design.
- **Notice of electronic disclosure.** Patients must be told their health information is disclosed electronically (a posted notice at the facility or on the website is enough for the general case), and outside treatment, payment and operations each electronic disclosure needs the patient's authorization first.
- **Enforcement.** The Texas Attorney General brings the civil action. Penalties per violation: up to **$5,000** if negligent, **$25,000** if knowing or intentional, **$250,000** if the information was used for financial gain, and up to **$1.5 million a year** for a pattern or practice. Licensing boards can put a licensed entity on probation, suspend it, or for an egregious pattern revoke its license, and a pattern finding excludes the entity from state-funded programs.
- **The selling point.** For a disclosure to another covered entity for a permitted purpose, the yearly penalty is capped at **$250,000** if the court finds the data was **encrypted**, or the recipient did not use or release it, or the entity had **security policies and employee training** in place. Encryption at rest and in transit, and a training record, are the first two things in the specification, and this is why. Good-faith compliance efforts also count as mitigation.
- **New since September 2025 (HB 4224, section 181.105).** Every covered entity must post, on its website and at the facility, instructions for a patient to request their records, contact the licensing board, and file a complaint. **That posting duty is the center's**, not RGF's; back-office processors working for a provider are exempt. It goes on the center's opening checklist.

Every number above is verified against the official statute text (Texas Legislative Council copy of Health and Safety Code chapter 181, current through the 2025 session; Sherlock, 2026-11-03). Say them plainly. The slide and the source line name the chapter.

BEAT

Each constraint in a sentence or two, then move. Volunteering them is what separates you from twenty years of vendors.

SLIDE 21 · The question you are going to ask · 29:00 to 30:30

Never cut this slide.

SAY

"The question you are going to ask is what happens when I get hit by a bus. Here is the answer, in order.

You own the specification. It is written for a competent engineer who has never met me, and it is the deliverable of the design engagement whether or not I build anything.

You own the code. It lives in a repository under your organization's account. You can revoke my access.

One distinction, and I would rather raise it myself than have you find it in a redline. What I am selling you is a scope of work: a design for one imaging center, built so it can be put anywhere. Everything specific to this center is yours outright, in your repository, and so are your configuration and your data. Some of what I would build is existing product of mine that I would extend for you. Who owns which pieces is settled in the contract, not at this table, and you keep running all of it either way. You are never hostage to me.

Runbooks, not memory. Every operational procedure is written down: how it runs, how it fails, who to call at each vendor.

No proprietary runtime. Standard interfaces, HL7 version 2, DICOM, payer APIs, and a named model vendor under its own business associate agreement. Any integrator can take it over.

Buy where buying is right. The RIS and the voice vendor are contracted in your name. Nothing routes through me that does not have to.

The design engagement exists so that this question has a boring answer. If I disappear in month four, you have a specification you can hand to anyone. If I disappear in year two, you have the code, the runbooks, and the vendor relationships in your name."

DEPTH / IF ASKED

The three tiers, so you can name them if he asks what exactly is his. One, the specification: theirs outright. Two, everything specific to this center, meaning configuration, workflows, integrations, data mappings, and reports: theirs outright, in their repository, your access revocable. Three, existing product of yours that you extend for them, such as the prior auth engine, the backfill engine, and the reporting core: yours, and how the center uses it is written into the contract. Say no more than that about the third tier at the table; it is the one that makes the price possible.

No license terms at the table (Rick, 2026-11-03; "perpetual" was already out 11-02). You are selling a scope of work, a design for one imaging center that can be put anywhere, and the contract settles ownership of any product pieces. Do not describe any arrangement of Luis's, with RadAssist or otherwise, in any form.

IF ASKED "Who owns what?" Short answer, then stop: "The contract settles it. The center owns its configuration and its data, and everything built specifically for this center. Where existing product of mine is part of the build, the contract says how you use it. We write that together, and you see it before you sign anything." If pressed further: "Not at lunch. That is a contract conversation, and I would rather get it right on paper than guess at it here."

Why raise it unprompted: he has negotiated these contracts, and he will find it in the redline anyway. Finding it yourself, out loud, at the table, is the difference between a negotiator and a vendor. It is the same move as slide 22.

If he asks why the product pieces are not simply his: "Keeping my own product mine is part of what keeps the price where it is. If you want a different arrangement, that is a contract conversation, and I am open to it."

Escrow: "Direct ownership is better than escrow. The code for your center is in your repository, so there is nothing to escrow. Whatever of mine the center runs is covered in the contract, which is the thing escrow is supposed to give you after a fight. You have it on day one, without the fight."

A second person: "There is no second person. That is the honest answer, and it is why the engagement is structured so that the specification and the runbooks are the continuity plan, not me."

Why this slide may matter more than any number: his bar for AI may be low. His bar for vendors is high, and he has been left holding systems by vendors before. Compensate with verifiable structure, not warmth. The ninety-day gate on slide 25 is what makes this promise financial rather than reassuring: he can stop.

Ask here, and write the answer down: "We are two people. If you were in my chair pitching you, what would you want to see about continuity and about getting out, before you would put us anywhere near your schedule?" Whatever he names is your path to a yes.

BEAT

Five points, in order, no skipping. The ownership distinction rides inside point two; say it there, in your own words, not at the end as a footnote. Then the "boring answer" line. Then the question. Then listen.

SLIDE 22 · The most important slide · 30:30 to 32:00

Never cut this slide.

SAY

Say the first line slowly.

"What I recommend you do not buy from me.

A custom voice agent. Buy a vendor. I will specify, evaluate, and integrate it, and I will tell you when it is not good enough in Spanish.

Reminders, self-scheduling, and intake forms. Your RIS does this. Make sure the contract says so, then use it.

Anything built to resell to other centers. I am not building a product to sell to other centers off the back of your build. I will say plainly that I keep the rights to my own product, and that is how the price stays where it is. What you get is a commitment about you. This center is the reference implementation. You are the priority. And if I ever take on a second site, your service terms do not change, and that is in writing.

Anything that touches the image clinically. Not from me, not from anyone, without an FDA pathway."

DEPTH / IF ASKED

The third bullet is printed on the slide as "Anything built to resell to other centers." An earlier draft printed a one-word heading that is on the never-say strip; it stays off the screen and out of your mouth, even in the negative. The reason the bullet exists at all: he has heard "and then we expand it" from everyone who ever pitched him, and he knows it means the person is about to become unavailable to his center. The spoken line is built around that. It makes no claim about your business model. It makes a commitment about his service level, in writing.

Why the narrow truth and not a flat no: a flat "no" would be false, because the product pieces stay yours under the slide 21 structure and you may well run them for another site one day. He will see that in the redline. A flat "yes, and I will serve other centers too" is the sentence he is allergic to. The honest middle is a commitment about him rather than a claim about you, and it happens to be the better deal for him: a reference implementation gets the most attention, not the least.

If he asks "is this going to be a product you sell to other centers": "Not off the back of your build. I keep the rights to my own product, and what your center runs is settled in the contract. What I will not do is make you one logo on a page. This center is the reference implementation, you are the priority, and your service terms are fixed in writing no matter what I do next. You know what the other version of that sentence means for your service level, so I am not going to say it."

If he or Luis asks "could you just build our RIS": "No, and I will tell you why. Your RIS is a system of record. If it stops, your center stops. That needs a vendor with a support organization and an uptime history, which is not one person. My value to you on the RIS is on your side of the table: picking it and negotiating the terms so you own your own data. I would rather be the person who makes sure you can get your data out of it than the person who holds it." Do not hint at a RadAssist RIS today. It is logged for after this engagement in BACKLOG-POST-ENGAGEMENT-2026-10-06.md, and raising it here would put you on both sides of the slide 3 contract advice.

If he asks about electronic health records, or whether you are building one: "No. An imaging center's record environment is the RIS plus the PACS. The referring physicians' EHRs are outside systems. You receive orders from them and send results back to them, over HL7 version 2 messages, orders and results, ORM and ORU in practice, plus DICOM for the images. That is the whole integration surface, and it is exactly what the four contract terms on slide 3 protect."

If he says "our last vendor promised the moon": "I know. That is why this slide exists. I am telling you to buy two of the things you asked about from someone else. The things I am proposing to build are the ones where the evidence is strongest and the vendors are built for someone much larger than you."

BEAT

Recommending against yourself is the single most credible thing you can do in front of someone who has sat through twenty years of vendor decks. Slow down.

SLIDE 23 · If it were my money · 32:00 to 33:00

Cut this slide second if you are running long (the full skip order is at the top of Part 2). Keep the closing question even if you cut the slide; ask it on the Thank You.

SAY

"If it were my money, I would do three things before signing another vendor contract.

Put the four access terms into every contract this month. It is free.

Decide now whether the prior authorization desk is a hire or a system. That decision expires the day you hire for it.

And tell me what front desk and prior authorization actually cost you last time, as a share of operating expense. Your number beats any benchmark I could bring.

One question for you. What would you do differently this time, knowing how the last one ended up?"

Then stop and listen.

DEPTH / IF ASKED

The third bullet is the most valuable question of the day. There is no defensible published benchmark for administrative labor as a share of imaging center operating cost. Everything circulating comes from content farms and financial-model templates. His P&L is the only number that matters, so ask for it and write it down.

⚠ **The second bullet, "a hire or a system," is where you must volunteer the staffing guardrail. Do not wait to be asked.** The bullet as written invites exactly the conclusion you do not want: that they should hold off hiring because a system is coming. Add this immediately after saying it:

> "And to be clear about what I am not saying. Staff for the manual workflow at opening. That is the only thing either of us can verify on day one, and I am not going to tell you to hire thin against a date of mine. What I will give you in Phase 1 is the sequence: which roles change as each piece goes in, and in what order, so each one is a decision you make later rather than a commitment you make now. Planned, attrition and role changes absorb most of it. Discovered, it becomes a layoff."

Why this is not a throwaway. Every other question in this deck is about Rick's schedule. This is about **their irreversible decision.** A center that opens with six front-office FTEs because the automation was not ready has made a hiring commitment that automation later converts into layoffs, in El Paso, in a small market, with Fernando's name on it and Luis practicing locally. Layoffs at a six-month-old imaging center is a story that follows both of them. It cuts the other way too: under-hiring against a promise that slips means a front office that cannot answer the phone during the exact window that forms their referral reputation. Both failure modes cost more than being wrong about a build date.

Volunteering it is also the single most credible thing available at that table. Same mechanism as the data-quality caveat: naming the limit of your own pitch is what makes the rest believable. **Fernando specifically will hear it,**

because he is the one who would have to do the firing.

FRAMING, NON-NEGOTIABLE. Never say automation **reduces headcount.** Always say automation **changes what your people do.** For a greenfield center scaling from zero that is also the *truer* claim: no existing job is eliminated, they are avoiding hires otherwise needed around month eighteen. Growth absorption, not reduction. Same economics, no layoff implication, and accurate. Scan your own phrasing for "replace," "eliminate," "reduce staff," and "FTE savings" and do not let any of them out of your mouth.

The staffing plan is now a named deliverable: Phase 3, "Architecture, Workflow and Staffing Plan" (slide 25). It is framed, on the slide and in your mouth, as how each role's work runs with the automation in place, so they hire for what the center actually needs on day one. It is never a headcount target. Luis knows how Fernando thinks about payroll; if Luis flags it before Thursday, the row stays and the wording is his.

Do not ask whether Southwest X-Ray was his. That is confirmed, and asking a question he has already answered reads as not listening.

The closing question is open on purpose. He is opening in direct competition with a company that owns what he built, and the buyer had financial trouble afterward. If his answer goes anywhere near the sale, the buyer, or how his time there ended: stop talking, listen, say little, ask nothing, move on. His answer tells you what he actually wants from this center, and it is probably control.

BEAT

Ask the closing question and then do not fill the silence. A man with his history has a long answer to it. Let him give it.

SLIDE 24 · What the dates depend on · 33:00 to 34:30

SAY

"The build starts around March so prior authorization works before you open. That holds if these things are true. A signed agreement. Your vendors chosen, one pick per system, with sandbox access to their interfaces. The vendor contracts and the business associate agreements signed before the build starts, and the hosting option picked by the end of the design phase. Test access to the payers' connections, and your payer enrollment started early enough that you can bill the day you open. One named person on your side who approves. Your expected volumes and payer mix before I design anything payer-specific. And the equipment delivery dates.

If one of these slips, the dates move with it. You hear about it the day it happens."

DEPTH / IF ASKED

This is the only slide that promises a date, and every date on it is conditional. **Added 2026-11-03 (Rick):** the list now carries the dependencies the engagement assumptions create: one pick per system, contracts and BAAs signed before Phase 4, the hosting option chosen by the end of Phase 3, and payer enrollment and credentialing started early (enrollment commonly runs months; a center that opens unenrolled cannot bill). The same list is on slide 25, the leave-behind and the runbook, worded the same way. The one most likely to slip is vendor selection, because it runs on their calendar: "The RIS choice drives everything after it. That is why the vendor work comes first."

Do not promise anything beyond the list. If pushed for a guarantee: "Tell me which piece has to be live by when, and we will scope it against these."

Staffing, unprompted: "Staff for the manual workflow at opening. I am not going to tell you to hire thin against a date of mine." It changes what your people do. It does not set your hiring plan.

BEAT

Read the list, then the last line, then stop. It is in the no-numbers deck too.

SLIDE 25 · What I am proposing: one engagement, five phases, $45,000 · 34:30 to 36:30

Never cut this slide if the number is being shown. It comes after the decisions and before the Thank You (Rick, 2026-11-03), so the number lands with the case already made, at about 34:30.

SAY, exactly this, once

"One engagement for the new center. Forty-five thousand, fixed fee, fixed scope, in five phases from now to May. Discovery, seven. Vendor selection, eight. Design and the staffing plan, seven. The build, fourteen. Testing, training and go-live, nine. Each phase ends in a document you sign, and the next one does not start until you have."

Then the payment line, plainly:

"Four thousand when we sign and three when you have the requirements. After that each phase is paid when it starts, except the build, which is seven on the first of March and seven on the first of April."

Then stop talking.

The five phases, so you can answer any row without looking

#	Phase	When	Deliverable you sign	Fee	Paid
1	Discovery and requirements. How the new center runs: patient flow from scheduling to report, services and modalities, volumes, payers, what is automated at opening. Luis's current center is an input, not the template.	Nov to mid-Dec 2026, 6 weeks	Requirements and Roadmap: requirements for every system, patient-flow map (order to signed report to paid claim), vendor contract terms to demand, make-or-buy recommendation for each requirement (met by a system you buy, or a piece I build, with a ballpark running cost for anything built), vendor shortlist and evaluation criteria for Phase 2	$7,000	$4,000 at signing, $3,000 on delivery
2	Vendor and system selection. (1) scheduling; (2) RIS and PACS, evaluated both as one bundled cloud system and as separate best-of-breed RIS plus PACS (Exa is one example); (3) prior authorization and eligibility; (4) patient phones, IVR and AI answering; (5) radiologist voice recognition and dictation; (6) billing and clearinghouse; (7) imaging-specific: modality integration and worklist, reading workstations, image sharing (CD and cloud). Demos, scoring, recommendation; interface fees fixed up front.	Mid-Dec 2026 to Jan 2027, 6 weeks	Vendor Selection Report: side-by-side comparison of the shortlisted systems in each category, a weighted scoring matrix against the Phase 1 requirements (integration readiness a named criterion), pros and concerns for each, and a 5-year total cost of ownership per option (license or subscription, implementation, interface fees, support, hardware), with a recommendation; plus the list of contracts to sign	$8,000	In full at the start
3	Design and staffing plan. System and network design, workflows, security and HIPAA plan, and how each role's work runs with the automation in place.	Feb 2027, 1 month	Architecture, Workflow and Staffing Plan, plus Total cost of ownership and operating budget: three hosting options (your cloud account; hybrid with a box at the center; all on-site with a local AI model), pros and cons, one-time, monthly and three-year cost including support labor; the owner picks	$7,000	In full at the start
4	Build and configure. Systems set up and connected, RadAssist configured, prior-auth automation, IT install coordinated. Depends on Phases 2 and 3 and on vendor lead times.	Mar to Apr 2027, 2 months	Every system, bought and built, set up, connected and passing end-to-end tests. Vendors configure their own products; I manage them, connect everything and run the tests	$14,000	$7,000 on March 1, $7,000 on April 1
5	Testing, training and go-live.	May 2027, 1 month	Signed go-live readiness, plus opening-week support	$9,000	In full at the start
	Total. Ready for a June 2027 opening, about a month of buffer.	Nov 2026 to May 2027		**$45,000**	

Outside the phases: $150 an hour, approved first, against a retainer the client tops up. The timeline flexes if the opening moves; the price does not. Do not compare the retainer to how their lawyer bills (Rick, 2026-11-02).

Assumptions, in one breath (Rick, 2026-11-03): "Five assumptions behind that number. It is priced for one center, with one pick per system; another center, or a different pick, I quote before I start. Running costs after the build are

not in the forty-five; they are billed to you at cost, in your own accounts, estimated in Phase 1 and budgeted in Phase 3. If the opening date moves, the timeline moves and the price does not. Anything outside the five phases is approved by you first, at a hundred and fifty an hour. And the dates hold while the list on the assumptions slide holds: contracts and BAAs signed before the build, the hosting option picked by the end of design, and your payer enrollment started early enough to bill on day one."

Those are the same assumptions, in the same words, on slide 25, slide 24, the leave-behind and the runbook.

DEPTH / IF ASKED

"**Why do we pay at the start of each phase?**" "Because you have already signed off the phase before it. You are never paying for something you have not seen. You are paying for the next piece after you approved the last one. The sign-off is your control, not the check." The largest single check is nine thousand. Phase 1 is the exception on purpose: four to start, three when the requirements are in your hands, so the first decision is small.

"**What if the opening slips?**" "The timeline moves with it and the price does not. The phases are the work; the dates are where the work lands. If June becomes August, the bars slide right and the buffer gets longer. I would rather you open ready than open on a date."

"**What does the $150 an hour cover?**" "Only things you ask for that are not in the five phases. You approve each one before I start, and it draws against a retainer you top up. If you never ask, you never pay it. Nothing in the five phases is ever billed by the hour."

If he pushes on price: shorten the term, never cut the rate. "Take Phase 1. Seven thousand, six weeks, and you own the requirements whether or not we go on." That is the gate. You never discount. Do not break out hours for the phases, do not justify the number at length, and never say what forty-five works out to per hour.

"**Does that change at three centers?" (Rick, 2026-11-03; this replaces the old answer.)** "The engagement covers this center, and one pick per system. If the other centers use the same picks, they're a smaller engagement: mostly build and go-live. If a center wants something different, that's its own evaluation, and I'll quote it before I start. You'll never get a surprise bill. You'll get a number to approve." The rule behind it: same picks at another center is a smaller follow-on, mostly Phases 4 and 5; different picks is a new selection for those categories, quoted and approved in writing before work starts.

"**What does it cost to keep running?" (Rick, 2026-11-03.)** Not in the forty-five, and say so before he asks. "Everything that keeps it running, the cloud, the AI, the texting, the clearinghouse, runs in accounts in your name. You sign each HIPAA agreement, you see each bill, I don't mark any of it up. I'll give you an estimated range in Phase 1 and a firm operating budget in Phase 3, and after opening you get a one-page statement every month: each vendor's bill, the volume it handled, what it did for you." If he wants a number today: "At expected volume it is around two thousand a month in vendors, with a cap I can't pass without your OK. That is an estimate, confirmed in Phase 1." Slide 31 carries the ranges. Where the money goes, if asked: a properly locked-down HIPAA cloud setup, the clearinghouse transactions (often already inside the billing contract), texting under a HIPAA agreement, security monitoring and a yearly penetration test; the AI usage itself is the small line. Frame it against staff time, never against "AI": one prior authorization coordinator costs more a month than the whole vendor stack. Opening-week support is inside Phase 5.

"**What happens after opening?" (support plan, APPROVED by Rick 2026-11-03 17:54.)** "After opening, there's a support plan: a flat monthly fee for a set number of hours, so you always know who to call and what it costs. If you'd rather have me on the team, we can talk about that too." The plan, if he wants the number: "Three thousand a month for up to twenty hours, a hundred and fifty an hour beyond that, starting when you open in June, for twelve months with a check-in at ninety days. It covers monitoring, fixes, updates to what I built, coordinating your vendors, and a response-time promise in writing. It is separate from the forty-five, and the running costs stay at cost." The row is on

slide 25 and the support line on slide 31. Keep "or a role on your team" as the alternative; no salary, no title (see the full-time block below).

"**Where does it run?" (the TCO deliverable, Rick, 2026-11-03.)** The Phase 3 deliverable "Total cost of ownership and operating budget" prices three options and the owner picks: A, the center's own secure cloud account; B, hybrid, a box at the center that strips names and IDs before anything goes to the AI company, with the database and backups in the cloud; C, everything on site including a local AI model. Say where you lean, and say the privacy part precisely: "Your patients' details do go to the AI company, the same way they go to your clearinghouse: under a signed HIPAA agreement, and never kept or trained on. If you'd rather names never leave, we can strip them first. Keeping the AI entirely at the center costs a lot more over three years and gives you a weaker assistant, so I'd only try it as a test." Never say "no data leaves." Stripping names reduces what the AI company sees; it does not remove patient information, because the packet has to carry the clinical story. Only a model running at the center keeps it from an AI company, and even then the payer, the clearinghouse and the backups still receive it.

If he asks what the billing evaluation is: "Billing is a buy and I am not touching the claims work. What I evaluate in Phase 2 is whether the billing vendor's eligibility and claim data are reachable, because the prior authorization system depends on them, and whether the four access terms go into that contract too."

Contract through RGF Global Ventures LLC, never personally. Do not discount because Luis is a friend; RadAssist is not inside this fee.

IF ASKED "Would you come in full time?" (Rick, 2026-11-03). They may float a salaried role. Say exactly this, and no numbers: "I'd want to hear it. Let's finish Phase 1 first; that's the job to judge me on. If it makes sense after that, we can talk about what a dedicated role looks like." Do not name a salary, a start date, or a title, and do not say whether you would leave anything else. If pressed: "Phase 1 is six weeks. Judge me on it, then we talk." Then back to the phases.

BEAT

Say it once. Stop. Let the silence sit. If he waves you on, the timeline is the next slide.

SLIDE 26 · The seven months, on one page · 36:30 to 37:30

This is the slide Fernando will study. He is visual. Walk it with your finger, left to right, once.

SAY

"We start in November with discovery, six weeks, and you sign the requirements before Christmas. Vendor selection runs through January, because the RIS choice drives everything after it. February is the design and the staffing plan. March and April is the build, and it cannot start until the vendors are chosen and the design is signed. May is testing, training, and the go-live sign-off. That leaves about a month of buffer before a June opening."

Then the point of the picture, once: "Every diamond is a document you sign. Every phase starts only after the diamond before it. The dollar marks are where the payments fall."

Then stop and let him look.

DEPTH / IF ASKED

Why the build waits on two things. The signed design, and the vendors' own lead times: contracts, sandbox access, equipment delivery dates. Say it plainly: "The build date is the one I control least, because part of it is on your vendors' calendars. That is why selection is Phase 2 and not Phase 4."

"**What is on the critical path?**" (Jesse's question.) Vendor selection and vendor lead times. Everything else is inside your control.

"**Can you compress it?**" "The phases can overlap a little once the requirements are signed, but I will not promise a shorter calendar than the vendors can deliver. The month of buffer is yours; I would rather hand it back unused than need it."

"**What if the opening slips to August?**" "The bars slide right. The work does not change and the price does not change."

BEAT

Finger on the opening flag. Stop.

SLIDE 27 · What I need from you · 37:30 to 38:30

SAY

One sentence: "This is what each phase needs from you, so nothing on my side is ever waiting on a question I should have asked earlier."

Then, to Jesse by name (Rick, 2026-11-03): "Jesse, I'd especially want your time on the technical and integration decisions."

Read the rows only if asked:

- **Phase 1:** time with all three of them, Dr. Ramos, Fernando and Jesse (never list only Luis and Fernando; Rick, 2026-11-03); the services and modalities they plan to offer; expected volumes and payer mix; the target opening date. Luis's current center is an input, not the template.
- **Phase 2:** budget ranges, any preferred vendors, the radiologists' dictation preference, and their decisions as the scoring comes in. What they get back: the Vendor Selection Report: side-by-side comparison of the shortlisted systems in each category, a weighted scoring matrix against the Phase 1 requirements (integration readiness a named criterion), pros and concerns for each, and a 5-year total cost of ownership per option (license or subscription, implementation, interface fees, support, hardware), with a recommendation; plus the contracts to sign for every system on slide 30.
- **Phase 3:** operating hours, the roles they intend to hire, the vendor contracts as they are negotiated, and their hosting pick from the three options. What they get back: the Architecture, Workflow and Staffing Plan, plus the Total cost of ownership and operating budget.
- **Phase 4:** signed vendor contracts and business associate agreements, sandbox access to each system, equipment delivery dates. What they get back: every system, bought and built, set up, connected and passing end-to-end tests; vendors configure their own products, I manage them, connect everything and run the tests (R26).
- **Phase 5:** their staff, hired and available for training, and the final opening date.
- **Throughout:** one named person on their side who approves.

DEPTH / IF ASKED

The line that matters to Fernando: Luis's current center is an input, not the template. This is a new center, designed from its own requirements. Luis's experience tells us what works and what to avoid; it does not decide the design.

Staffing, if it comes up here. The Phase 3 plan is how each role's work runs with the automation in place, so they hire for what the center actually needs on day one. It is never a headcount target. Say "changes what your people do," never "reduces." If he wants a number, say you will have one in February, from the requirements, not today.

"**Who approves on our side?**" Ask them to name one person today if they are ready; otherwise it is the first question of Phase 1.

BEAT

The Thank You is next; it is the last slide of the pitch. (If the room asks for the shortlist or the running costs afterward, slides 30 and 31 are in the appendix; then close the laptop.)

SLIDE 28 · Thank you · 38:30 to 40:00, then stop selling or turn to the appendix

SAY

"Every one of these decisions is reversible today. None of them will be in six months.

Thank you, all three of you. Here is a summary you can keep."

DEPTH / IF ASKED

Contact on the slide: Richard Finlay, RGF Global Ventures LLC, (480) 532-1367, rfinlay813@gmail.com.

BEAT

Put the leave-behind on the table, one per person (two pages: the second is the agent explainer, the dates, and the pricing). This is the last slide of the pitch, and it now comes after the engagement, the timeline and what I need from you (Rick, 2026-11-03), so the number has already landed. If slide 23 was cut, ask its closing question here. Then stop selling and finish lunch; the appendix (27 to 33) is reference, only if the room asks. In the no-numbers deck this slide follows slide 24 directly.

SLIDE 29 · Appendix: reference, only if asked · 40:00, appendix, only if the room asks

SAY

"The rest is reference. Stop me here if you would rather take it another day."

DEPTH / IF ASKED

This divider exists so the room can stop before the reference material. The appendix is 30 (the Phase 2 systems and starting shortlist), 31 (running costs and the three hosting options), 32 (the opening checklist), 33 and 34 (ideas worth a look) and 35 (radiology report assistance, the two directions and the across-centers default). In the no-numbers deck only 30 and 35 remain after this divider (Rick, 2026-11-03: the engagement, timeline and what-I-need slides, 25 to 27, are the ones removed, along with 31 to 34).

BEAT

One sentence. Watch him. If he waves you on, advance to whichever slide answers his question. If he does not, stop.

SLIDE 30 · Systems to select in Phase 2: starting shortlist · appendix, only if asked

Reference. Show it only if someone asks which systems or which vendors. It is a starting shortlist from a landscape review, not an evaluation; the final pick is Phase 2 with their radiologists in the demos.

SAY

"This is where I would start looking, not what I would buy. Phase 2 is where your radiologists sit in the demos and we score it."

Then, if he wants the position: "For a center your size I'd lean toward one cloud system for RIS and PACS: one vendor, fewer interfaces, faster opening. The contract is what keeps you free to leave. Picking the vendor is Phase 2."

On integration (Rick, 2026-11-03): "I score vendors on how easily they connect, not just features, and I write the integration terms into the contract."

DO NOT SAY: never name a winner or quote an integration score or ranking at lunch. That is Phase 2 work. The internal readiness review (`SHERLOCK-INTEGRATION-READINESS-2026-11-03.md`) is reference only and nothing from it goes on a slide or in the leave-behind.

DEPTH / IF ASKED

The slide, row by row. RIS and PACS: evaluated both as one bundled cloud system and as separate best-of-breed RIS plus PACS; the bundled candidates built for independent centers are the starting names are Konica Minolta Exa PACS|RIS, RamSoft OmegaAI, Novarad and Intelerad. The 2026 Best in KLAS PACS winners, Sectra and Agfa, are built and priced for hospitals; "small" in that award still means a community hospital's volume. A premium viewer with a separate RIS only if the radiologists insist, at a higher cost and one more interface. What decides it: a hands-on reading demo with their radiologists on MRI and CT cases; five-year cost including interfaces, storage growth and dictation; front desk to claim in one workflow; hosting and security (BAA, SOC 2, backup and recovery, uptime); export and API terms; two or three references from single-site centers of similar volume; a timeline that lands before June, which means a contract by about February.

Dictation: the radiologists choose the tool and the owners choose the contract. The candidates are the RIS's built-in dictation (cheapest, no interface), PowerScribe One (now Microsoft), Fluency for Imaging (now Jacobian; 2026 Best in KLAS for imaging speech recognition) and Rad AI. If the group is contracted rather than employed, it may already carry a license and only need the link.

Billing and clearinghouse: outsource billing to an imaging-specialized firm at opening, on a short contract, with the clearinghouse and the data in the center's name and clear handover terms. Clearinghouses in common use: Waystar, Availity, TriZetto, Office Ally, Quadax (2026 Best in KLAS). Document a second clearinghouse path; the 2024 Change Healthcare outage is the lesson. What decides it: three quotes; TRICARE, Medicare and Medicare Advantage handling; eligibility and authorization transactions reachable by the Phase 4 build.

Also selected in Phase 2: scheduling and reminders (inside the RIS); prior authorization and eligibility connections; patient phones, IVR and AI answering, which is the patient-facing system and a separate row from the radiologists' dictation; modality integration and worklist; reading workstations; image sharing (CD and cloud). Each carries the four contract terms from slide 3, and the interface fees are fixed up front.

Do not quote a license price, a per-study rate, a market share, or an interface fee; none is sourced. "Every price in this market is a quote, which is why the shortlist goes to RFP." Never disparage a vendor on the slide. If Luis or Fernando already lean toward one, write it down; it gives Phase 2 a front-runner, not a decision.

BEAT

One line, then let him read. If he names a vendor he has used, ask what he liked and what he would not sign again. That is Phase 2 input.

SLIDE 31 · What it costs to keep running, and where it runs · appendix, only if asked

Reference. Show it if he asks what it costs after opening, or where the data lives. Every dollar on it is an estimate, confirmed in Phase 1.

SAY

"Running costs are not in the forty-five. Everything runs in accounts in your name. I hold the admin keys, you hold the ownership. If you ever let me go, nothing stops."

"I don't mark any of it up. You see the actual bills every month on one page, with what each one did for you."

"At expected volume it is around two thousand a month in vendors, with a cap I can't pass without your OK. That is an estimate; Phase 1 confirms it and Phase 3 turns it into an operating budget you approve."

THEN SAY (the privacy line, precisely)

"Your patients' details do go to the AI company, the same way they go to your clearinghouse: under a signed HIPAA agreement, and never kept or trained on. If you'd rather names never leave, we can strip them first. Keeping the AI entirely at the center costs a lot more over three years and gives you a weaker assistant, so I'd only try it as a test."

DEPTH / IF ASKED

The three options on the slide. A, the center's own secure cloud account: keeps working through a power or internet outage at the center, no new duties for staff, the best available model; the clinical content goes to the AI company under the agreement. B, hybrid: a box at the center swaps names and IDs for tokens before the call and the database and backups live in the cloud; names never reach the AI company, but the clinical story still does, so the agreement is still required; two environments to patch; the agents pause in an outage and catch up; the staff lock the room and keep a log. C, all on-site with a local AI model: no AI company sees patient data, no per-use cost; but the models that fit on hardware at the center are weaker at the long, rule-heavy work (matching a payer's criteria, drafting an appeal), so quality has to be proven on test cases first; it stops in an outage; the center carries the full physical-safeguard duty; the hardware is obsolete in three to four years. Where you lean: A, with B's name-stripping step run inside the cloud account if they want identifiers kept away from the AI company; B's box only if the RIS forces a local connection; C not for opening, offered later as a measured side-by-side test.

The ranges, all estimates at about 650 studies a month: vendors about $900 to $4,200 a month in A, expected near $2,000; about $1,000 to $4,300 in B; about $1,200 to $3,500 in C plus the hardware. One-time setup outside the fee: $6,000 to $25,000 in A (penetration test, clearinghouse enrollment, interface onboarding), $12,000 to $37,000 in B, $27,000 to $52,000 in C. Where the money goes: the locked-down HIPAA cloud setup, the clearinghouse transactions (often already in the billing contract, so ask the billing vendor first), texting under a HIPAA agreement (the line most likely to surprise), security monitoring and a yearly penetration test. The AI usage itself is a small line even at 900 studies a month.

The support row (Rick, 2026-11-03 17:54): $3,000 a month for up to 20 hours, $150 an hour beyond, from June 2027, 12-month term, 90-day check-in, separate from the $45,000. Vendors plus support at expected volume is about $5,000 a month in the cloud option. Do not quote a three-year total; Phase 3 produces it.

Never say "no data leaves." Stripping names reduces what the AI company sees; it does not remove patient information, because a prior authorization packet has to carry the clinical story. Only a model running at the center keeps it from an AI company, and even then the payer, the clearinghouse and the encrypted offsite backup still receive it.

BEAT

Three lines, then stop. If he asks for the cap, say it is set with him in Phase 1 from the real volumes, not today.

SLIDE 32 · Opening checklist: required to open, outside the fee · appendix, only if asked

Reference. Six things the center must do to open and bill. None is Rick's build; none is in the fee; each gets a named owner in Phase 1.

SAY

"None of this is my build and none of it is in the forty-five. It is what you have to do to open and bill, and in Phase 1 every line gets a name and a date, because the first one takes months."

Then the one that matters: "Payer enrollment and credentialing is the longest lead item on this list. If it starts late, you open and cannot bill. It starts in Phase 1."

DEPTH / IF ASKED

The six rows: payer enrollment and radiologist credentialing (Medicare IDTF enrollment, Texas Medicaid, TRICARE, commercial payers, CAQH; center administrator with a credentialing service; starts in Phase 1); ACR accreditation for MRI and CT (Medicare requires it to bill the technical component; brings CT dose records, an MR safety policy, physician peer review and a physicist QC program with it; medical director with a contracted physicist; plan in Phase 1, submit after go-live); Texas DSHS CT registration (registration, shielding, dosimetry, operator credentials; physicist and administrator; starts in Phase 1); CLIA certificate of waiver (before the first fingerstick creatinine; administrator); HIPAA security basics (risk analysis, MFA, endpoint protection, backup and downtime plan, incident response and Texas breach notice; designed in Phase 3, owned by the center); HB 300 staff training (within 90 days of hire, signed statements kept, plus the posted records-request notice; administrator, as staff are hired).

Say "months" for enrollment; do not quote a day count. The owner roles on the slide are proposals: ask who on their side owns each one and write it down. Compliance-only systems (dose monitoring, MRI safety screening, contrast and injector logs) are bought inside the RIS or PACS or from a dose tool and are never pitched as value.

BEAT

Point at row one. Ask who owns it. Write the name down.

SLIDE 33 · Ideas worth a look · appendix, only if asked

Reference. Not in the fee. Scoped and priced only if they ask, after Phase 1.

SAY

"When your radiologist says 'repeat in six months,' we make sure it happens and the referrer knows. That protects the patient, protects the radiologist, and brings the follow-up back to you."

THEN SAY (the caveats, unprompted)

"Those numbers come from hospital programs, most of them with a person making the calls, not just software. And the outreach always says 'your doctor recommended' and goes through the referrer. We never ask a patient for a scan directly."

DEPTH / IF ASKED

Follow-up recommendation tracking: every report that recommends more imaging is logged, the referrer is told, and at the due date someone checks it happened; if not, one outreach call. Tracked programs completed follow-up 78% versus 52% untracked (a nurse-navigator lung-nodule program, Respir Med 2022); a private 33-radiologist group's own tool took lung-nodule follow-up from 26.5% to 59.7% (BMJ Open Qual 2019). Never quote a malpractice dollar figure; none is sourced. A Rick build on top of report delivery, a few weeks, estimate only.

Good-faith cost estimates at scheduling: across 470,422 outpatient imaging visits at UC Irvine, patients given a staff-generated out-of-pocket estimate had nearly twice the odds of completing the exam (OR 1.91, Clin Imaging 2025). An association, not a trial; patients who ask for estimates may differ. Rides on the eligibility feed, satisfies the No Surprises Act in the same step; mostly a RIS or billing-vendor feature.

One-liners: structured referrer orders (at Emory only 51% of abdominal CT requisitions carried the cancer history; "if the order arrives complete, prior auth is mostly paperwork"; the link to denials is inference, not a sourced number); peer learning instead of score-based review (accreditation requires peer review anyway; peer learning took radiologists less than half the time in one study, Clin Imaging 2024; bought, not built); contrast pre-screening plus fingerstick creatinine ("a two-minute fingerstick, not a second appointment"; needs the CLIA waiver on slide 32; bought).

BEAT

Lead line, caveats, stop. If Luis lights up at the follow-up tracker, write down what he would want it to do first. That is a requirement, not a reaction.

SLIDE 34 · Ideas worth a look, continued · appendix, only if asked

Three smaller ideas, each bought or configured, not built. One line each.

SAY

Structured referrer orders: "If the order arrives complete, prior auth is mostly paperwork; if it doesn't, it's phone calls, so we make the order carry what the payer will ask for."

Peer learning: "Accreditation requires peer review anyway; we set it up as peer learning, which takes radiologists less than half the time."

Contrast screening: "A patient who shows up for a contrast CT without a recent kidney test should get a two-minute fingerstick, not a second appointment."

DEPTH / IF ASKED

Structured orders: at Emory only 51% of abdominal CT requisitions carried the cancer history (PMID 40517119); the link to denials is inference, so say "fewer chase calls," never a denial number; mostly a RIS referrer-portal feature, the field list is Phase 3 work. Peer learning: 0.76 versus 1.71 hours a month per radiologist (Clin Imaging 2024, PMID 38113549); a PACS module or the ACR program, nothing to build. Contrast: the kidney questions go into the pre-visit intake, the fingerstick needs the CLIA waiver on slide 32; a device and cartridges, bought. Dose monitoring, MRI safety screening and contrast logs are compliance items, never value.

BEAT

One line each. Then close the laptop.

SLIDE 35 · Appendix: radiology report assistance, the two directions · appendix, only if asked

Appendix since 2026-11-03 (Rick). Show it only if someone asks which direction "requesting records" meant, how the report reaches the RIS, or how more than one center would be deployed. The one line that stays in the pitch is on slide 18. The direction question, if you want it settled, is asked on slide 18 too.

SAY

"You asked about records, so here is the piece of this that already exists and runs every day, and the two directions records can mean.

The agent drafts. The radiologist signs. Nothing clinical leaves this building without a human signature.

That is RadAssist today, and Luis just told you what it is like to use. Calcium reads the scanner's numbers from a screen capture Luis pastes; thyroid and spine take what he graded, one dropdown per sentence, and draft the report

from his own template. He confirms every value before anything applies, and he signs. At your center the path has a name: DICOM worklist and query for the study, HL7 version 2 ORU to carry the signed report into the RIS. Both are contract terms from slide 3.

Requesting records runs the other way, and it is a different build. DICOM query and retrieve where a facility grants it. A request out and a DICOM import where it does not. And from January, the payer's Provider Access API for the claims record, which is in-network only and carries documents, not images.

Which one did you mean? They are two different builds, and I would rather ask than guess."

DEPTH / IF ASKED

Why this slide exists: your 10-10 email to Luis said "your partner asked about scheduling, answering patients, and requesting medical records." In a radiology practice, records has two directions, and the deck has to make Luis pick. Slide 15 is the records request seen from the other side; this slide is the report, and the honest line between the two.

RadAssist, stated exactly and no further: calcium scoring, thyroid ultrasound (ACR TI-RADS), Spine MRI (cervical, thoracic, lumbar; live since 2026-10-31), and CT Spine (live since 2026-11-02; the same tool switched to CT, with the bony canal, calcification and CT disc fields, built from his sample reports). For calcium, Luis snips the scanner's calcium report or the dose screen, pastes it, and RadAssist reads per-vessel scores, total, age, sex, and DLP, shows every value with a confidence mark, and applies nothing until he clicks Apply. For thyroid and spine he enters what he graded and the impression writes itself in the order he set. It drafts from his own template. No patient data is stored on the server; the inputs are computed in the browser. It does not read DICOM, does not touch the PACS, and does not interpret an image. The viewer at his current site is server-side rendered and never gives the browser a DICOM object, which is why the screen-capture path exists at all. Do not claim it does more than that today; Luis is sitting there and will correct you.

The named path at a center they own, and why it is honest to name it: DICOM Modality Worklist and query/retrieve give the report assistant the study context, which is contract term four. HL7 v2 ORU is the standard result message every RIS sends out; what has to be verified before signature is that the RIS also accepts an inbound ORU result, because Konica Minolta's Exa, for one, documents only outbound report paths. That is a Phase 1 evaluation item. Say "I will verify that against the RIS you pick," never "it will work."

Records coming in: DICOM query/retrieve across organizations is rare because the sending facility has to grant it; otherwise the request goes out by fax or portal and the images come back on a disc or a link and are imported over DICOM. The payer Provider Access API under CMS-0057-F arrives 2027-01-01, carries the claims and clinical record the payer holds, never images, and requires an in-network contract plus a treatment relationship with the patient, which a newly opened center may not have on day one. Name that limit before he does.

The FDA line, if he connects this to slide 19: the radiologist's own numbers, the radiologist's own template, the radiologist's signature. It formats what the scanner already measured. Anything that would recommend a follow-up interval or flag growth is on the other side of the wall and is declined.

Deployment across centers (one environment, isolated instances): the default is in the last block of this slide, below. The slide survives either answer. Do not say as fact that the centers share anything, and do not name the other principals; their spellings are unconfirmed.

Not on the scorecard, deliberately. The scorecard is the list of candidates. This is not a candidate; it exists and Luis uses it. Extending it to the new center is Tier 3 product, and who owns what is settled in the contract, same as the prior auth engine.

If asked how long: no date. "Which direction, and against which RIS. Then I can scope it."

BEAT

Say the heading slowly, then let Luis answer the direction question. Write down which one he says. That answer changes the Phase 3 design.

THEN SAY, LAST ON THIS SLIDE, AND ONLY IF MORE THAN ONE CENTER COMES UP (the one-stack question)

"One specification, written once and reused for every center. The way I would deploy it is the industry standard, and it is security first. One environment, under one contract, which is what keeps the cost down. Inside it, each organization gets its own isolated instance: its own database and its own encryption keys, its own credentials with role-based access, and its own audit log. Nobody at one organization can see another's data, by construction. If one owner or one joint venture owns all the centers, that default is the whole answer. If the centers are separately owned and an owner wants fully separate hosting and separate books, each gets a fully separate stack, and that costs more. Either works. What I will never build is one shared database."

Why the default runs that way, if he asks: the isolation lives at the data layer, where it has to be, so one environment can be operated, patched and monitored once while no organization shares a database, a key, or a login with another. Fully separate stacks add nothing for security that the isolated instance does not already give; they add separate hosting bills and separate books, which is exactly why a separately owned center sometimes wants them. If he asks whether one shared database with a center column would be cheaper still: "Cheaper, and I would not build it. The boundary belongs in separate databases and separate keys, not in a filter." If he asks whether reporting can ever span the centers: "Later, as an explicit decision by all the principals, as a read-only view, never a merged record." Do not state as fact which case they are in. Ask.

PART 3: REHEARSAL DRILLS

Drill 1: the five headline numbers

Cover the right column. Say the number, then say its caveat, from memory. Then check.

Slide	The number	The caveat you must say unprompted
4	4.1 million denied. 11.5% appealed. 80.7% of appeals overturned. Four in five win, almost nobody files.	Medicare Advantage, all services. Not an imaging number, because CMS does not collect prior auth data by type of service. The only audited, payer-reported, national figure that exists.
5 and 8	13 hours a week. About 40 requests per physician per week. 40% have staff working exclusively on it. You have not hired that person yet.	AMA 2025 survey, a thousand physicians. Self-reported.
8	At least 220 million hours and 16 billion dollars over ten years. Based on AMA data showing 14 hours per week.	CMS's projection in the rule. Not measured savings. Nobody has gone back and counted.
12	UC Irvine, about nineteen thousand exams. 22.3% cancelled, 2.0% no-show. Seventy percent of cancellations patient-initiated. Backfill, not reminders.	One academic center, 2022 and early 2023. Your numbers will differ. The pattern will not: cancellations outnumber no-shows, most with notice.
13	Mass General, about seven thousand MRI patients, text on top of a phone call. 5.1% to 3.8%. No change in punctuality.	1.3 points absolute. Vendors say 25%. Same study. Only two thirds of the texting group reachable by text. Your RIS will do this. Let it.

Second pass: the sources, one line each, from memory.

Slide	Source
4	KFF, 2026-01-28, from data insurers report to CMS
5 and 8	AMA 2025 Prior Authorization Physician Survey
8	CMS-0057-F, 89 FR 8758, Regulatory Impact Analysis
12	Aijaz et al., Acad Radiol 2024, PMID 38705764
13	Liu et al., J Am Coll Radiol 2017, PMID 28583321

Drill 2: the price answer

The single most rehearsed line in the deck. Say it out loud ten times. Then say it once more and stop talking for a full ten seconds afterward. The stopping is the part you are practicing.

"One engagement for the new center. Forty-five thousand, fixed fee, fixed scope, in five phases from now to May. Discovery, seven. Vendor selection, eight. Design and the staffing plan, seven. The build, fourteen. Testing, training and go-live, nine. Each phase ends in a document you sign, and the next one does not start until you have."

Then the payment line: "Four thousand when we sign and three when you have the requirements. After that each phase is paid when it starts, except the build, which is seven on the first of March and seven on the first of April."

If pushed: "Take Phase 1. Seven thousand, six weeks, and you own the requirements whether or not we go on."

The three objections, one line each: **why upfront per phase** ("you have already signed off the phase before it; the sign-off is your control"), **what if the opening slips** ("the timeline moves, the price does not"), **what does $150 an hour cover** ("only things you ask for outside the five phases, approved first, against a retainer you top up"). Two more, added 2026-11-03: **three centers** ("this center and one pick per system; same picks elsewhere is a smaller engagement, mostly build and go-live; anything different is quoted before I start") and **running costs** ("not in the forty-five; accounts in your name, at cost, a monthly statement, around two thousand a month at expected volume with a cap").

No hours. No rate. No justification. Shorten the term, never cut the rate.

Drill 3: the bus answer, five points in order

1. You own the specification. Written for a competent engineer who has never met me. The deliverable whether or not I build anything.
2. You own the code. Center-specific code in a repository under your organization's account. You can revoke my access. Then the distinction, raised by you, before the redline: I am selling a scope of work, a design for one center that can be put anywhere; everything specific to this center, its configuration and its data are yours outright; who owns which product pieces is settled in the contract, not today, and you keep running all of it either way. No license terms at the table.
3. Runbooks, not memory. How it runs, how it fails, who to call at each vendor.
4. No proprietary runtime. HL7 version 2, DICOM, payer APIs, a named model vendor under its own BAA. Any integrator can take it over.

5. Buy where buying is right. The RIS and the voice vendor are in your name. Nothing routes through me that does not have to.

Closer: "The design engagement exists so that this question has a boring answer."

Escrow: direct ownership beats escrow for the center's code, and whatever of mine the center runs is covered in the contract, which is what escrow is for, without the fight. Who owns what: the contract settles it; the center owns its configuration and its data. Why the product stays mine: it is part of what keeps the price where it is; a different arrangement is a contract conversation. Second person: there is no second person, and that is why the spec and the runbooks are the continuity plan.

Other centers, if it comes up here instead of at slide 22: not off the back of your build. This center is the reference implementation, you are the priority, and your service terms do not change if I ever take on a second site. In writing.

Drill 4: the questions you ask them, in the order you would ask them

Write every answer down at the table.

1. **At the open, before anything about his history:** "Luis told me you built and sold imaging here in El Paso before this, but I would rather hear it from you than secondhand. What did you build, and how many centers were you running at the end?"
2. **Slide 2, before the four contract terms:** "Which RIS and billing vendors are you looking at, and have you signed anything yet?"
3. **Slide 5, the 40% statistic:** "How many prior authorizations a day were you running at steady state, and who did them?"
4. **Slide 9, the federal dates:** "What is your payer mix going to look like?"
5. **Slide 12, before you lean on 22.3%:** "What was your cancellation rate, and what did you do with the empty slot?"
6. **Slide 14, before the voice recommendation:** "Who takes the phone call when a patient describes chest pain?"
7. **Slide 16, owner reporting:** "What do you want to see on one page every Monday morning?"
8. **Slide 19 or 20, the boundaries:** "You came up as an RPA, so you have been in the room with the patient. When you think about automating the front end, where is the line for you? What has to stay human?"
9. **Slide 21, the bus question:** "We are two people. If you were in my chair pitching you, what would you want to see about continuity and about getting out, before you would put us anywhere near your schedule?"
10. **Slide 23, the close:** "What did front desk and prior authorization actually cost you last time, as a share of operating expense?" Then: "What would you do differently this time, knowing how the last one ended up?"

The runbook numbers these 1 through 8 with two items each numbered 5 and 6. There are ten questions. Ask all ten.

Drill 5: the 60-second version

If lunch collapses to one minute, this is what survives. Say it as one paragraph.

"Here is what I would tell you if I had one minute. You are in the only window where automation is a design choice and not a retrofit. Before you sign any vendor, put four terms in the contract: API access to your own data, HL7 interface fees fixed at signature, export rights at no charge, and DICOM query permissions. That is free today and hostage terms later. The biggest cost you have not planned for is prior authorization. In Medicare Advantage, four in five appeals win and almost nobody files one, because nobody has time. I would build that, and I would tell you to

buy your reminders from the RIS and your phone system from a voice vendor, not from me. You own the spec, you own the code for your center, who owns what else is settled in the contract, and if I get hit by a bus you hand all of it to anyone. Forty-five thousand to take the new center from requirements to opening day, in five phases, each paid when it starts and each started only after you sign off the one before. You can click through a working demo today."

Drill 6: the 5-minute version

Cut slide 16 first, then slide 18 to two sentences. Never cut 6, 21, 22, or 25. In five minutes you keep:

Minute	Slides	What you say
0:00 to 0:45	1, 3	Framing line. The four contract terms, fast. "Whatever you decide about me, do this."
0:45 to 1:45	4, 5	Four in five, stop. Caveat. Forty percent, you have not hired that person yet. Caveat.
1:45 to 2:30	7, 17	"The thing you did not ask about." Then the scorecard in one breath: two of the things you asked about, I am telling you to buy.
2:30 to 3:15	18	RadAssist in two sentences: Luis uses it daily, his templates and rules, it drafts and he signs, nothing leaves the browser. "Luis, one Tuesday." Thirty seconds of Luis.
3:15 to 4:00	21	The five bus points, in order. "A boring answer."
4:00 to 4:30	22	What I recommend you do not buy from me. Slowly.
4:30 to 5:00	25	The price, once. Stop.

What is lost in the 5-minute version: every caveat except the two on slides 4 and 5, the federal dates, the records rules, the boundaries slides. If he asks about any of them, you have the full answer above.

Fernando Q&A (rehearsed 2026-11-03)

Ten questions, asked by Gio as Fernando on 2026-11-03; these are the approved answers, verbatim. Say them as written. Q11 to Q16 are Rick's approved lunch lines from his evening review the same day; they sit before Q10 so the closing question stays last.

> **Patterns to hold.** State the fixed price plainly: forty-five thousand, five phases, priced today; never hedge it. Answer first, then stop; no second paragraph unless he asks. Name specifics: Phase 1 at seven thousand, the demo he will click, the contract terms, the runbook.

Q1. "You're one guy. Why trust you instead of buying what everybody else buys?"

"Fair question, and you should ask it. I'm not going to tell you what to buy. Where something on the market fits, I'll tell you to buy it and help you pick it. Where it doesn't, I'll build it to your requirements.

On trusting one guy: three things. Luis uses software I built on real patients every day. You'll click through a working demo in a few minutes. And every phase is a fixed fee that you sign off before the next one starts, so if I don't deliver

Phase 1, you walk away having spent seven thousand dollars, not forty-five."

Q2. "You get hit by a bus or take a job. Who fixes it at 7 a.m.?"

"Fair. At 7 a.m., the front desk is still scheduling, because scheduling runs in the system you buy, with that vendor's support line. Nothing I build sits in the way of a patient getting booked. If my piece goes down, your staff do it by hand, the way they would anyway.

And if I'm gone: you own everything I build. The code lives in your account, and you can revoke my access. Every piece has a specification a competent engineer can pick up, and you get a runbook that says, case by case, who to call, the response time, and how to reach them."

Q3. "$45k before a single patient. Why now, not after opening?"

"You're not spending forty-five thousand now. You're spending seven, for Phase 1, and then you decide. Here's why now: the cheapest moment to get your data terms right is before you sign with a RIS or PACS vendor. Data export, interface fees, API access. Once the contract is signed, you've lost that leverage, and fixing it later means renegotiating or rebuilding. Waiting until you're open doesn't save money. It means paying to redo decisions you could have made for free."

Q4. "If AI writes the code, why not hire a kid from UTEP with the same AI?"

"Good question. Yes, I use AI, the same way your radiologist uses a scanner. Anyone can buy the same scanner. You're paying for the person who knows what the image means. The AI writes code fast. It doesn't know what an imaging center needs on day one, which contract terms protect your data, what Texas privacy law requires of a vendor, or when the AI is confidently wrong. That's eighteen years of running technology programs, and it's why the software Luis uses every day works. A kid from UTEP with the same tools would get you code. You'd still need someone to tell him what to build, and to catch what he gets wrong."

Q5 (Jesse). "Where does patient data live, who can see it, including the AI company?"

"Three parts. Where: in the United States, always. Texas law has required that for patient records since January. Every vendor we pick has to show it in writing, and so does anything I build. Who: only the people whose job needs it. Access is by role, every view and change is logged, and your center sits in its own isolated environment, with its own database and keys. No other organization can see it. The AI company: only under a signed business associate agreement. That makes them legally bound under HIPAA, they don't train on your data, and the AI only sees what a task needs. Anything clinical still gets a human signature before it leaves the building."

Q6. "What if in March the build takes longer and costs more?"

"You won't see that movie with me, because the price is fixed. Forty-five thousand, five phases, each one priced today. The build is fourteen thousand, and it stays fourteen thousand. If the date moves, the timeline moves, not the price. If you want something new that isn't in scope, I tell you the cost first, you approve it in writing, and only then do I do it. And you sign off each phase before the next one starts, so you're never more than one phase in."

Q7. "Why fixed fee? How do I know you didn't pad it?"

"Because with hourly, every surprise is your cost. With a fixed fee, it's mine. If a phase takes me longer than I planned, you don't pay a dollar more. On padding: every phase is priced separately, with the deliverables listed. You can see exactly what seven thousand buys in Phase 1. And you only pay for one phase at a time, after you've signed off the last one. If I padded it, you'd see it in Phase 1 and walk away."

Q8. "Luis is your neighbor. How do we keep business clean with a friendship in the middle?"

Never call Luis your RadAssist partner at this table.

"Same way you'd want it with anyone: on paper. A written contract, a fixed scope, a price per phase, and your sign-off before each phase starts. If something goes sideways, we go to the contract, not to the dinner table. And Luis isn't the referee. You and I deal with the business directly. That's what keeps the friendship out of it. If it isn't working, you stop after a phase, and nobody has to have an awkward conversation over the fence."

IF ASKED about RadAssist and Luis: "Luis uses software I built, and he's shaped it as a radiologist. Any arrangement between us on that is separate, and it's in writing. It isn't part of this."

Q9. "What do you need from us, and what do we get at end of Phase 1? I don't pay for slides."

"To start, I need four things: a few hours with all three of you, Dr. Ramos, Fernando and Jesse, and Jesse, I'd especially want your time on the technical and integration decisions; the services and modalities you plan to offer; your expected volumes and payer mix; and your target opening date. Four thousand at signing. Six weeks later you get five documents you own: the requirements for every system the center needs; a map of how a patient moves from order to signed report to paid claim; the contract terms to demand from every vendor before you sign; a make-or-buy recommendation for each requirement, met by a system you buy or a piece I build, with a ballpark running cost for anything built; and a vendor shortlist with the evaluation criteria, ready for Phase 2. Then three thousand on delivery. And you own all of it whether we go on or not. If you stop there, you've got a requirements package any vendor or consultant can work from."

Q11. "What RIS and PACS would you pick?" (Rick, 2026-11-03)

"For a center your size I'd lean toward one cloud system for RIS and PACS: one vendor, fewer interfaces, faster opening. The contract is what keeps you free to leave. Picking the vendor is Phase 2. And in Phase 2 you get every shortlisted system scored side by side with its five-year cost, so the pick is yours with the numbers in front of you."

Q12. "What if each center wants different systems?" (Rick, 2026-11-03)

"The engagement covers this center, and one pick per system. If the other centers use the same picks, they're a smaller engagement: mostly build and go-live. If a center wants something different, that's its own evaluation, and I'll quote it before I start. You'll never get a surprise bill. You'll get a number to approve."

Q13. "What will it cost to run?" (Rick, 2026-11-03)

"Running costs are billed to you at cost, in accounts you own. In Phase 1 you get a ballpark, and in Phase 3 a total cost of ownership with three hosting options side by side, so you choose with the full picture."

Q14. "Where will it run? Cloud or here?" (Rick, 2026-11-03)

"I'd run it in a secure cloud account the center owns, with a business associate agreement. If you'd rather have hardware in the building, I can do that too, and I'll show you the cost of each."

If Jesse presses on whether the AI company sees patient data: "Yes, and I won't tell you otherwise. Your patients' details do go to the AI company, the same way they go to your clearinghouse: under a signed HIPAA agreement, and never kept or trained on. If you'd rather names never leave, we can strip them first."

Q15. "What is this Texas law you keep mentioning?" (HB 300; Rick, 2026-11-03)

"Texas privacy law covers me directly, not just through our contract. If I mishandle your patients' data, I'm the one the state comes after. That's how it should be."

Q16. "What happens after opening?" (Rick, 2026-11-03)

"After opening, there's a support plan: a flat monthly fee for a set number of hours, so you always know who to call and what it costs. If you'd rather have me on the team, we can talk about that too."

Q10. "One reason, one sentence, why say yes today."

"Because the decisions that are free today stop being free the day you sign your first vendor contract, and finding out if I'm right costs you one phase, not the whole engagement."

Curveballs (Rick, 2026-11-03)

Short answers, one breath each. Answer, then stop.

"What if Luis leaves the partnership?"

"The contract is with the center, not with Luis. You own everything I build: the specification, the code, the runbooks, the data. Nothing in it depends on one person, including me."

"Who else have you done this for?"

"Luis uses software I built every day. Before that I took a $250 million, 2-million-square-foot distribution center with its own data center from an empty site to opening day: triple-redundant telecom from the road to the main network room, every vendor for security, cabling and equipment, and all the IT. This center is the next one."

Guardrails: no go-live claims for any enterprise software program by name; "clearance eligible" is the only clearance wording, and only if asked. The $250 million project is a 2-million-square-foot distribution center with its own data center (Arlington, Tennessee greenfield build); never call it a data center (Rick, 2026-11-03 17:57).

"Can you guarantee the opening date?"

"No one honestly can, and I won't. Every date I've shown you is conditional on the list on the assumptions slide. What I can guarantee is the price: it's fixed. The timeline flexes; the number does not."

PART 4: PREP PLAN, TUESDAY TO THURSDAY MORNING, 2026-11-05 (Rick-approved 2026-11-03, updated 21:13)

Lunch is Thursday 2026-11-05 with Luis, Fernando and Jesse. What is not on the list waits. There is no RadAssist demo setup: slide 18 is spoken (Luis, 2026-11-03).

Tuesday 2026-11-03, all done

- [x] **Rehearse the prior authorization demo end to end** (slide 10), on the laptop and on the phone, both banner states.
- [x] **Mock Q&A as Fernando with Gio, about 20 minutes** (approved answers in "Fernando Q&A" at the end of Part 3).
- [x] Royalty decision (Option A), the support plan ($3,000 a month, up to 20 hours), and the full review of the deck and notes through the final structure round.

Wednesday 2026-11-04

- [ ] **Review the updated slides with Luis, recorded.** Ask him: does the slide count feel right for an hour's lunch; who on their side owns each line of the opening checklist (slide 32); anything not to say in front of Fernando or Jesse. His read on how Fernando hears the price is the point.

- [ ] **Send Gio the recording.** Gio applies ONE last update round, renders, syncs OneDrive and Drive. Then the documents freeze.
- [ ] **Full mock Q&A with Gio, about 40 minutes, Fernando AND Jesse:** every Fernando question, Q1 to Q16 plus the curveballs (end of Part 3), then a full Jesse technical round (data, security, integration, critical path). Every question, not a sample.
- [ ] **Wednesday night, full practice run out loud, recorded:** slides 1 to 28 against the clock, pricing at about 38 minutes. Send the recording to Gio. Then a **short second run** of the opening (1 to 3), the demo transition (9 into 10) and the close (21 to 28).

Thursday 2026-11-05, morning

- [ ] **Full mock Q&A with Gio again, 30 to 40 minutes, Fernando AND Jesse, all questions** (Q1 to Q16, curveballs, the Jesse technical round). Early enough that the demo check and printing still happen before Luis arrives.
- [ ] **Text Luis the demo link to forward to Fernando and Jesse.**
- [ ] **Open the demo on the phone (cellular) and on the laptop; check the banner; reload.** Leave both tabs up.
- [ ] **Print the leave-behind** (two pages, three copies) **and the lunch card** (`Imaging-Center-LUNCH-CARD-2026-11-05.pdf`, two pages). Bring this sheet too, folded, Part 1 face up.
- [ ] Luis picks you up. Lunch runs about an hour.

Not before Thursday

- **Luis's CT confirmation.** It comes when it comes; do not push.
- **The royalty and licensing conversation.** Private, with Luis only, after the lawyer's review. Not in the room, not on the drive over.

Already done

- Demo-steps email sent to Luis. He replied that the overview only is what he wants; applied everywhere (slide 18 is spoken).
- The partner name is in (2026-10-06). The opening window is ~June 2027, April at the earliest (2026-10-22). The engagement is $45,000 in five phases, approved 2026-11-02; every red tag is gone.
- The deck runs with no wifi; the demo is the only thing that needs a network. Leave the Sherlock research document at home; everything you need from it is in this sheet, the deck notes and the RUNBOOK.

Open items: ALL FOUR RESOLVED 2026-10-06

These were found while building this sheet and have since been fixed in the deck and the RUNBOOK. Listed so you know what changed and why, not because anything is outstanding.

- **The 40-minute clock now closes.** It used to give the "what not to buy" slide the final minute and allocate nothing to the rest of the close, while the RUNBOOK crammed seven slides into three minutes. Fixed by folding the CMS slide into the AMA slide (one argument, told once) and taking two minutes off the RadAssist demo. **The close now owns 36:00 to 40:00**, with the engagement getting 38:30 to 40:00 from the appendix. The deck was

reflowed to 24 slides on 2026-10-06 and to 26 on 2026-10-14 (report assistance at 18, appendix at 25 and 26), and every slide number in this sheet has been renumbered to match.

- **"A platform" is off the screen.** That bullet heading now reads "Anything built to resell to other centers," which makes the same promise without printing a never-say word in large type behind you.
- **The "two centers" error is corrected** to one company with five El Paso locations, in the deck presenter note where it lived.
- **The RUNBOOK objection answer no longer quotes "and then we scale it."** It names the consequence instead of the slogan.

The one judgment call left is yours, and it is not a defect: if the lunch runs long, follow the skip order at the top of Part 2 (16, then 23, then 13 to one sentence, then 8 to its headline and last line, then 18 to two sentences), and never slides 21, 22, or 24.

Deck order is narrative, on purpose

The slides run in the order that moves Fernando from skepticism to a decision, which is why the free move comes at slide 3, before you ask for anything, and the price comes last. That is not the order of value. **Do not re-sequence the deck by value.** The ranked view, highest value to lowest, lives on the leave-behind, which is the artifact he keeps. Items 6 and 7 on that list ranking low is slide 22 and is the point. Item 1 stays at the top even though you earn nothing from it, which is what makes the list believable.

PART 5: GLOSSARY AND ACRONYM BUSTER

Every acronym and term of art that appears in this sheet, in alphabetical order. Each entry is the expansion, then one line on what it means for this deal. Nothing here is a statistic; the numbers stay on their slides.

ACS. American Community Survey. The Census Bureau's ongoing survey; the source of the El Paso limited-English figure on slide 14.

AMA. American Medical Association. Its 2025 physician survey is the source of the 13 hours and 40 percent figures on slides 5 and 8. Self-reported survey data, which is why the caveat is mandatory.

API. Application programming interface. A documented way for one system to read or write another system's data without a human in a screen. Contract term one on slide 3; the payer-side APIs on slide 9 are what the prior auth agent consumes.

ASTP / ONC. Assistant Secretary for Technology Policy, formerly the Office of the National Coordinator for Health IT. The federal office that sets health data exchange policy; the RSNA letter on slide 15 was addressed to it.

BAA. Business associate agreement. The HIPAA contract a provider signs with any vendor that handles patient data on its behalf. Every vendor in this design needs one, including the AI model vendor, and slide 20 names what the AI vendor's agreement does and does not cover.

BLS. Bureau of Labor Statistics. Source of the El Paso wage figure on slide 5.

CAQH. Council for Affordable Quality Healthcare, now publishing as DataSpring. Its annual index is the source of the 63 percent of plans already building the prior auth API on slide 9. Payer self-report.

CFR. Code of Federal Regulations. Where the TCPA healthcare exemption and the HIPAA records rules actually live. Citing the CFR section means you read the regulation, not a summary.

CHIP. Children's Health Insurance Program. The state-run coverage for children in families above the Medicaid line. Covered by the federal prior auth rule alongside Medicaid.

Clearinghouse. The intermediary that carries EDI transactions between a provider's billing system and the payers. Usually bundled with the billing vendor; it is where eligibility checks run.

CMS. Centers for Medicare and Medicaid Services. The federal agency that runs Medicare and oversees Medicaid, and the author of the prior auth rule. When the sheet says "CMS collects" or "CMS projected," it means this agency.

CMS-0057-F. The Interoperability and Prior Authorization Final Rule, published in the Federal Register in February 2024. The January dates on slides 2 and 9 come from it. It binds certain payers, not providers.

DICOM. Digital Imaging and Communications in Medicine. The standard for storing and moving medical images. DICOM query permission is contract term four on slide 3, and it is how the records tracker pulls a prior study from the PACS.

DICOMweb. The web-based version of the DICOM protocols, so images can be queried over ordinary HTTPS instead of older network protocols. Named in contract term four so the permission covers both.

EDI 270 / 271 / 278. Electronic Data Interchange transactions, the standardized messages that have run between providers and payers for decades. The 270 asks whether a patient is eligible, the 271 answers, and the 278 requests a prior authorization. The fallback path for commercial plans the federal rule does not reach.

EHR. Electronic health record. The patient's clinical record system. Texas SB 1188 on slide 20 requires electronic health records with Texas patient data to be stored in the United States.

Escrow (software escrow). An arrangement where a neutral third party holds a copy of a vendor's source code and releases it to the customer if the vendor goes out of business or breaks the contract. It is what a buyer asks for when he does not trust that the code will outlive the vendor. Slide 21 declines it because the center's own code already sits in their repository and the product pieces come with the source in their hands on day one, so there is nothing left for an escrow agent to hold.

FCC. Federal Communications Commission. The agency that writes and enforces the TCPA rules, including the 2024 ruling that an AI voice counts as an artificial voice.

FDA. Food and Drug Administration. The agency that regulates medical devices, including software that interprets images. Slide 19 is the statutory reason none of the five candidates is a device.

FHIR. Fast Healthcare Interoperability Resources, said "fire." The modern API standard for health data. The payer APIs required by the federal rule are FHIR APIs. On the RIS side, do not pay extra for it and do not plan around it; HL7 version 2 is what the RIS will actually speak.

FR. Federal Register. The daily journal of federal rules. "89 FR 8758" is volume 89, page 8758, where the prior auth rule begins.

FTE. Full-time equivalent. One full-time position's worth of labor. On the never-say strip: never frame any of this as displaced FTEs.

Greenfield. A center being built from nothing, with no existing systems, staff, or contracts to unwind. The entire argument of the day: every choice is still free.

HHS. Department of Health and Human Services. The federal department over CMS, FDA, and OCR. Its 2016 guidance is where the records-fee labor line on slide 15 comes from.

HIPAA. Health Insurance Portability and Accountability Act. The federal patient privacy and records law. Source of the 30-day records clock and the fee rule on slide 15, and the reason every vendor needs a BAA.

HL7 / HL7 v2. Health Level Seven, version 2. The decades-old messaging standard that a RIS uses to talk to a PACS, a billing system, or anything else. Every connection is an "interface," and vendors quote them one at a time, which is why contract term two on slide 3 fixes the count and the fee schedule at signature.

KFF. Formerly the Kaiser Family Foundation, an independent health policy research organization. Its analysis of payer-reported CMS data is the source of the four-in-five headline on slide 4.

MA. Medicare Advantage. Medicare coverage delivered through a private insurer under contract with CMS, instead of traditional Medicare run by CMS directly. The headline number on slide 4 is a Medicare Advantage number, and the federal rule binds these plans.

MCP. Model Context Protocol. A standard for connecting an AI model to outside tools and data. Named on slide 20 only because data flowing through third-party MCP connectors is outside the AI vendor's BAA.

MRI. Magnetic resonance imaging. One modality. The 4.94 percent denial rate on slide 8 and the Mass General reminder trial on slide 13 are both MRI-only studies.

Modality. An imaging method: MRI, CT, X-ray, ultrasound, mammography. Owner reporting on slide 16 is broken out by modality.

OCR. Office for Civil Rights, inside HHS. The office that enforces HIPAA, including the records access deadlines. An OCR complaint is the failure mode the records tracker exists to prevent.

ORM / ORU. The two HL7 version 2 message types that carry an order (ORM) and a result (ORU). How a referring physician's EHR sends the center an order and how the center sends the report back. Slide 22: the integration surface with outside EHRs, and the reason you are not building one.

PACS. Picture Archiving and Communication System. Where the images themselves are stored. The RIS tracks the order and the schedule; the PACS holds the pictures. Records retrieval is a DICOM query against the PACS.

Payers. The organizations that pay the center for a study: commercial insurers, Medicare Advantage plans, traditional Medicare, Medicaid and CHIP, exchange plans, and the patient when self-pay. Spelled out on slide 2. The federal rule reaches only Medicare Advantage, Medicaid and CHIP, and the exchange plans.

Payer mix. The share of a center's volume that bills to each payer type. The question on slide 9, because it decides how much the January date is worth to him.

PHI. Protected health information. Any patient-identifiable health data under HIPAA. Where PHI is present, the BAA and the Texas residency rule apply.

PMID. PubMed identifier. The reference number for a peer-reviewed article in the National Library of Medicine's index. Every study cited in this sheet carries one so it can be pulled up and checked.

Prior authorization. A payer's advance approval of a study before it is performed and billed. The largest cost the center has not planned for, and the lead build candidate.

Rate governor. The per-patient limiter in the messaging design that counts every reminder, backfill offer, and results notice against the TCPA cap of one a day and three a week. Slide 20: a send queue alone does not do this.

RIS. Radiology information system. The operational system of a radiology practice: orders, scheduling, patient registration, worklists, reporting, and usually reminders. The single most consequential vendor choice they will make in the next six months.

RPA. Radiology Practitioner Assistant. Fernando's clinical credential, not a software term: an experienced radiographer certified to perform fluoroscopic and invasive procedures and assess patients under a radiologist's

supervision, without interpreting images. The question in Drill 4 uses it on purpose, because almost no software vendor knows what it is. Not to be confused with robotic process automation, which does not appear in this sheet.

RSNA. Radiological Society of North America. The professional society whose March 2026 comment letter is the source for the prior-study retrieval gap on slide 15.

SB 1188 / HB 300 / HB 149 / AB 3030. Texas Senate Bill 1188, Texas House Bills 300 and 149, and California Assembly Bill 3030. State statutes named on slides 14, 19, and 20: US-only storage of Texas health records, the Texas privacy law that reaches Rick directly, the Texas AI disclosure obligation, and the California AI disclosure law that excludes scheduling and billing.

TCPA. Telephone Consumer Protection Act. The federal law, enforced by the FCC, that restricts automated calls and texts to consumers. Its wireless healthcare exemption lets a provider send appointment reminders and similar messages without separate written consent, but only within eight conditions, including one message a day and three a week per patient and no billing content. Slides 12, 14, and 20.

TEFCA. Trusted Exchange Framework and Common Agreement. The federal framework for nationwide health data exchange. Slide 15: it moves documents today but does not yet require image exchange, which is why priors still arrive on discs.

21 U.S.C. 360j(o). The section of federal law that lists software functions excluded from the medical device definition, including administrative support of a facility. The statutory answer on slide 19.

45 CFR 164.524. The HIPAA right-of-access regulation: the 30-day clock, the one extension, and what a records fee may include. Slide 15.

47 CFR 64.1200(a)(9)(iv). The TCPA wireless healthcare exemption and its eight conditions. Slide 20.